

THE ILLNESS MACHINE

How Social Systems Create,
Shape, and Sustain Mental Illness



— A POPULAR-SCIENCE INVESTIGATION —

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Author's Note

This work was created under the umbrella of Axiologic Research as part of two interconnected research programs, one dedicated to Artificial Intelligence and the other to Outfinitism, both led by Șinică Alboaie. To explore the details of how these two programs intersect and build upon each other, we invite readers to visit the Outfinity Venture Studio page at www.outfinity.ch.

While Artificial Intelligence language models were used to assist with text generation, the foundational philosophy, thematic structure, and analytical approach derive exclusively from the author's decades of dedicated study of social phenomena, social technologies, and genuine hard technologies resulting from various European and self funded research projects. Recently, within the Achilles research project, we have been deeply studying AI generated literature and its automated verification using neuro symbolic approaches; all the free books made available to the public are part of the suite of works we use to test our latest technologies.

Research edition, August 2026.

This book is a work of science communication. It does not provide diagnosis, treatment, or individualized medical advice.

Scientific claims are calibrated to the strength of the underlying design. Historical analogies and counterfactual political-system analyses are explicitly separated from direct empirical evidence.

Internet discussions are treated as qualitative material rather than epidemiological evidence. No identifiable post is quoted.

References were checked against scholarly and institutional sources available through August 2026.

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A Note on Evidence and Political Language

This book asks a question that is easy to turn into a slogan and difficult to turn into science: can social systems make people mentally ill? The word system is used here in a demanding sense. It means more than “other people” or “the environment.” It includes the organized patterns through which a society distributes money, safety, status, time, housing, work, recognition, punishment, care, and the right to define what counts as normal. A family can operate as a system. So can a school, a workplace, a hospital, a prison, a welfare bureaucracy, a city, a labor market, a digital platform, or a political regime.

The phrase mental illness is equally difficult. It covers conditions as different as major depression, post-traumatic stress disorder, schizophrenia, bipolar disorder, substance-use disorders, anxiety disorders, and the forms of distress that never receive a diagnosis. These conditions do not have one cause, one biological signature, or one relationship to the social world. A claim that may be well supported for trauma-related disorders can be too strong for bipolar disorder. A mechanism that helps explain the persistence of depression may not explain the first onset of psychosis. Throughout the book, “mental illness” therefore functions as a family of problems rather than a single object.

The evidence is organized by strength. Cross-sectional associations can reveal patterns but cannot determine which way causation runs. Longitudinal studies establish temporal order but remain vulnerable to hidden differences between people and places. Natural experiments, policy changes, sibling comparisons, genetically informed designs, and randomized interventions provide stronger leverage. None is perfect. A randomized housing program can tell us what happened to the people offered a move under a particular set of conditions; it cannot reveal what every city would do to every mind. A genetic study can demonstrate

inherited liability; it cannot show that social arrangements are irrelevant. The most credible conclusions emerge when different methods, with different weaknesses, converge.

The book also draws on public internet discussions. Posts in mental-health, survivor, antipsychiatry, and diagnosis-specific communities are not treated as epidemiological evidence. Their participants are self-selected, their identities are uncertain, and their most memorable stories are not necessarily the most representative. Yet such discussions are valuable qualitative material. They show how people interpret their own suffering, which causal stories they accept or reject, where services fail them, and why apparently reassuring clinical language can sometimes sound like an accusation. The recurring themes are used as questions for the research literature, not as substitutes for it.

Historical examples are handled with a similar distinction. The fact that psychiatry once classified homosexuality as a disorder, that Soviet authorities used “sluggish schizophrenia” against dissenters, or that nineteenth-century physicians invented racist diagnoses does not prove that all present diagnoses are political constructions. It proves something narrower and still important: psychiatric categories are produced inside institutions, and institutions can make grave errors when social judgment disguises itself as medical discovery. Conversely, the fact that diagnostic systems are fallible does not make hallucinations, mania, despair, terror, or suicidal pain unreal.

My conclusion is deliberately more precise than the title. Social systems can cause some mental disorders in some people; they can raise or lower population incidence; they can shape the content and meaning of symptoms; they can turn vulnerability into illness; and they can prolong illness by reproducing threat, humiliation, deprivation, isolation, stigma, and failed care. They are rarely sufficient causes, and they are never the whole explanation. Biology and society do not compete for possession of the patient. Social experience becomes biological experience, while

biological differences alter how social experience is perceived, survived, and remembered.

This is a work of science communication, not a diagnostic manual or a substitute for clinical care. Its argument is political only in the elementary sense that preventable suffering should matter to public decisions. The scientific standard remains adversarial: every attractive claim must survive the best available counterargument. The book asks the reader to resist two comforting errors at once. The first is that every wound has a private origin. The second is that changing society would abolish mental illness. The evidence supports neither innocence nor omnipotence. It supports responsibility.

Chapter Four deliberately compares systems for which the evidence is radically unequal. Nazi genocide, political imprisonment, forced institutional deprivation, colonial residential schooling, and Mao-era rustication leave historical populations that can be studied directly or quasi-experimentally. Nordic social democracy and liberal welfare capitalism can be compared only observationally across countries and policies. Libertarian minimal states, anarcho-capitalism, neoreactionary neocameralism, and the Venus Project have never existed in a sufficiently clean form to support psychiatric epidemiology. Where the book discusses these systems, it is performing constrained causal speculation: it asks how a proposed design would redistribute exposures already linked to mental health, such as material insecurity, social isolation, status competition, autonomy, voice, exit, and concentrated power. Those passages are hypotheses, not findings.

Political behavior is not psychiatric diagnosis. Envy, demagoguery, authoritarian submission, collective narcissism, identity fusion, conspiracy belief, leader worship, and ideological rigidity can be socially destructive without constituting mental illness. The phrase "regression to childhood" is used only as a historical or psychoanalytic metaphor. When the book makes an empirical claim, it uses more measurable constructs such as dependency, threat sensitivity, identity fusion, need for cognitive closure,

leader idealization, humiliation, and reduced epistemic autonomy. A society may be called pathological in a functional or moral sense, but that metaphor must never be confused with diagnosing its citizens.

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The Casino That Became a Mental-Health Experiment

In the early 1990s, researchers began following 1,420 children in the rural mountains of western North Carolina. They interviewed the children and their parents year after year, recording symptoms, family circumstances, and psychiatric diagnoses. About a quarter of the children were members of the Eastern Band of Cherokee Indians. The study was not designed as an experiment in social policy. Then a casino opened on the reservation.

The casino's profits funded annual payments to enrolled tribal members. Families did not receive the money because a therapist judged them motivated, because a caseworker approved a behavior plan, or because researchers randomly assigned them to an intervention. The payments arrived because of a change in the local political economy. Some families who had been poor moved above the poverty line. Others remained poor. The children had already been assessed repeatedly before the payments began, which gave the researchers something rare: a view of mental health before and after an income shock that was not caused by a child's symptoms or a parent's employability.

The results, published by epidemiologist E. Jane Costello and colleagues in JAMA in 2003, were precise enough to frustrate ideologues. Before the casino opened, children in families that would later escape poverty showed roughly the same elevated psychiatric symptom levels as children who remained poor. After the income supplements began, the ex-poor children's symptoms of conduct disorder and oppositional defiant disorder fell toward the levels of children who had never been poor.

Anxiety and depression did not show the same improvement. The intervention mattered, but not uniformly. Money did not function as a universal antidepressant. It changed particular pathways in particular children (Costello et al., 2003).

The casino study is famous because it resembles a controlled interruption of an argument that otherwise runs forever. Poor children have more mental-health problems. One explanation says that poverty harms them. Another says that parents and children with inherited or pre-existing vulnerabilities are more likely to become poor. Both processes can occur. The casino did not eliminate selection, but it weakened a central objection. The income change was not a reward for becoming psychologically healthier. It preceded the improvement. Later work on the same cohort suggested that the benefits extended into adulthood for some outcomes and that changes in family relationships helped transmit the effect. The social intervention entered the home as fewer conflicts, altered supervision, and a different emotional climate rather than as a molecule crossing the blood-brain barrier.

A story about a casino is an odd place to begin a book about psychiatry. That is exactly why it is useful. Mental-health policy usually starts after distress has become a patient. A person enters a clinic, answers questions, receives a formulation or diagnosis, and is offered treatment. The causal world is narrowed to what can be changed inside that encounter. The casino changed no diagnostic manual and prescribed no therapy. It modified a condition upstream of the clinic.

The natural experiment also prevents the book from becoming a sermon about inequality. The effects were not magical, and they were not the same for every disorder. Poverty can produce irritability, conflict, surveillance, instability, and harsh parenting without being the sole cause of childhood anxiety or depression. A serious social account must be disorder-specific when evidence permits, must acknowledge resilience, and must admit that many exposed people remain well while some unexposed

people become ill. “Everything is social” is not a scientific conclusion. It is a way of avoiding the work of specifying mechanisms.

Yet the study makes one proposition difficult to dismiss: changing a family's place in an economic system can change a child's psychiatric trajectory. That proposition is stronger than saying that poverty is “associated with” mental illness. It implies that at least some cases classified as disorders are partly produced by modifiable social arrangements. The symptoms are real. The children are not pretending. The diagnosis is not merely a label. The cause is nevertheless not contained inside the child.

Once this possibility is admitted, the borders of psychiatry become politically uncomfortable. If income can alter psychiatric symptoms, then a finance ministry makes mental-health policy. If job design can precipitate depression, a manager participates in prevention or causation. If a housing system prolongs psychosis by placing people in danger and isolation, zoning and rent policy enter the clinical record whether clinicians mention them or not. If algorithms amplify social comparison, harassment, sleep loss, and compulsive use, a recommendation system becomes an environmental exposure. If discrimination raises the risk of mental disorder, civil-rights enforcement is not adjacent to mental health; it is one of its instruments.

The converse is equally important. When systems improve, clinics do not become obsolete. The Cherokee payments did not eliminate every disorder. Foster care does not reverse all effects of deprivation. Housing does not automatically end hallucinations. A compassionate society will still contain bipolar disorder, obsessive-compulsive disorder, dementia, autism, addiction, trauma, psychosis, and despair. Human nervous systems are variable and vulnerable. Life contains loss even under good institutions.

The most defensible claim is therefore more disturbing than either extreme. Social systems do not create all mental illness, but they are part of the causal architecture of mental illness. They can generate exposure, distribute vulnerability, trigger onset, determine meaning, and sustain disability. They can also remove risk, interrupt feedback loops, and make

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recovery more likely. The question is not whether the brain or society is responsible. The question is how brains become ill inside worlds built by other people.

The Causal Question

The Dangerous Verb

In 1851, the American physician Samuel A. Cartwright proposed a diagnosis for enslaved Africans who ran away. He called it drapetomania. The disorder's defining sign was the desire to escape bondage. Cartwright treated resistance to an institution as evidence of pathology within the person resisting it. The diagnosis is now remembered as an obscenity disguised as medicine, but its logic did not disappear with its name. Every society retains the temptation to locate the defect in the person who cannot tolerate what the society has normalized.

This historical example is so grotesque that it can make the present feel innocent. Modern psychiatry does not diagnose workers with “refusal-of-the-factory disease” or debtors with “aversion-to-compound-interest disorder.” Diagnostic manuals explicitly require distress or impairment and contain safeguards against treating mere social deviance as illness. Yet the basic problem is more subtle than fraudulent diagnoses invented by racists or dictators. A person can have genuine symptoms and still be responding intelligibly to a damaging world. The symptoms can become biologically embedded, persist after the environment changes, and respond to medical treatment. Real illness and social causation are compatible.

The question “Can social systems create mental illness?” therefore contains several questions that are often collapsed into one. A system may increase the incidence of a recognized disorder by exposing people to violence, deprivation, humiliation, or uncontrollable stress. It may activate vulnerability in people who would otherwise have remained well. It may

shape the content of symptoms without causing the underlying vulnerability. It may prolong illness by denying housing, income, relationships, or humane care. It may define a behavior as pathological because the behavior threatens prevailing norms. It may also create the administrative conditions under which ordinary distress is converted into a diagnosis, or severe disorder is ignored because the affected person is considered morally defective. These are different mechanisms. Evidence for one does not prove the others.

Mental disorder itself is not a single natural kind. Major depression, schizophrenia, panic disorder, post-traumatic stress disorder, anorexia nervosa, obsessive-compulsive disorder, and substance-use disorders differ in their heritability, mechanisms, typical course, and relationship to social conditions. The categories are also syndromes: clusters of signs and experiences rather than diagnoses established by one biomarker. That does not make them imaginary. Migraine was real before its biology was understood, and pain remains real without a blood test. Psychiatric categories can be imperfect maps of genuine suffering.

The distinction between distress and disorder complicates the issue further. Grief after bereavement, fear during war, insomnia during eviction, and anger under abuse may be proportionate responses. Calling every painful response a disorder medicalizes adversity. Refusing the word disorder whenever adversity is involved creates the opposite problem. Trauma can produce disabling symptoms that persist long after danger ends. Depression may become self-sustaining even when it began in a recognizable loss. A response can be understandable in origin and pathological in duration or intensity. The fact that a fire explains a burn does not make the burn unreal.

The provocative sentence “society makes people mentally ill” contains at least three different claims, and arguments about it often fail because the speakers defend one while attacking another.

The weakest claim is distributional. Mental disorders are not evenly distributed. Rates vary by income, exposure to violence, migration status,

neighborhood, gender, racialization, employment, housing, and many other social conditions. This is now beyond serious dispute. The comprehensive review by Kirkbride and colleagues (2024) assembled evidence across the life course and concluded that social determinants contribute materially to the onset, course, and outcome of mental disorders. Lund and colleagues (2018) reached a similar conclusion in a systematic review of reviews linking mental health to the Sustainable Development Goals. Distribution, however, is only the beginning. A map of unequal suffering does not identify what produced the inequality.

The strongest claim is totalizing. It says that what we call mental illness is created by social norms, institutions, or power, and that biological disease is largely a category error or ideological disguise. Versions of this claim appear in antipsychiatry, critical psychiatry, disability scholarship, social constructionism, and online survivor communities, though these traditions differ greatly and should not be collapsed into one. The totalizing claim captures real episodes of medical abuse and diagnostic inflation. It also fails as a general theory. Mania does not vanish when an observer adopts a more tolerant vocabulary. Severe melancholia can appear in people with money, love, safety, and social approval. Psychosis occurs across cultures and historical periods. Rare genetic variants, polygenic liability, neurodevelopmental differences, pharmacological responses, sleep disruption, immune processes, and medical illnesses all supply evidence that at least some psychiatric syndromes cannot be explained as nothing more than society's judgment of deviance.

The defensible claim lies between these poles. Social systems can be components of causal mechanisms. They can increase the probability that a disorder begins, determine when a latent vulnerability crosses a clinical threshold, influence what symptoms mean and how they are expressed, and change whether an episode resolves or becomes recurrent. They can also alter the category itself by deciding which differences receive medical names. The verb create is therefore legitimate only when its object is specified. A system may create risk, create cases, create chronicity, create

disability, or create a diagnosis. Those are related but distinct achievements.

This precision matters because mental disorder is not identical to distress. Grief is painful without usually being a disease. Fear in a burning building is adaptive. Suspicion under persecution can be accurate. Exhaustion after impossible work may be an intelligible response before it becomes a syndrome. Yet an understandable response can still become disabling, self-sustaining, and in need of treatment. Calling a condition socially intelligible does not make it voluntary, trivial, or safe. The body does not suspend biology when the cause is meaningful.

Philosophers of psychiatry have long debated whether disorder requires an internal dysfunction, social harm, statistical rarity, failed agency, or some combination. Jerome Wakefield's influential "harmful dysfunction" account tried to separate the factual question of whether a mechanism is failing from the value-laden question of whether the failure is harmful. Critics noted that our judgments about evolved function are uncertain and that harm cannot be purified of culture. The debate does not yield a simple definition, but it exposes why psychiatric classification is unusually vulnerable to social influence. A broken femur remains broken under different moral regimes. The boundary between eccentric conviction and delusion, intense sadness and major depression, energetic temperament and hypomania, or identity and pathology depends partly on duration, impairment, danger, context, and expectations about ordinary life.

That dependence does not make diagnosis arbitrary. Blood pressure thresholds are also conventions imposed on continuous risk, and cancer categories change with improved knowledge. The special difficulty in psychiatry is that the organ being assessed is the organ through which a person interprets norms, relationships, and reality. The symptom and its context speak through the same mouth.

The causal debate becomes clearer when separated from the classificatory debate. A social process can cause a condition whose diagnosis is scientifically valid. War can cause post-traumatic stress disorder

without making PTSD a political fiction. Lead exposure can impair cognition even if the boundary of “impairment” is conventional. Conversely, a diagnostic category can be socially constructed while tracking genuine suffering. The category “major depressive disorder” is a human instrument; insomnia, suicidal despair, psychomotor slowing, and inability to experience pleasure are not inventions of a committee. Instruments can cut nature at useful joints without discovering a single essence.

The first great alternative to social causation is social selection, sometimes called social drift. People with emerging mental illness may lose educational opportunities, employment, income, housing, and relationships. They may move into poorer neighborhoods or be confined in institutions. If investigators arrive after these changes, poverty appears to precede nothing; it is the scar left by illness. This mechanism is especially important in schizophrenia. In a landmark study, Bruce Dohrenwend and colleagues (1992) compared ethnic groups with different socioeconomic trajectories and found evidence that selection played a larger role for schizophrenia, while social causation appeared more important for depression among women and for substance use and antisocial disorders among men. Later work has complicated the pattern, but the central lesson remains: the direction of causation can differ by diagnosis.

Selection is not a defeat for the social thesis. It often marks the beginning of a feedback loop. A psychotic episode may cause job loss; job loss may then worsen stress, isolation, medication access, housing, and relapse risk. Depression may reduce earnings; debt collection and shame may deepen depression. Mental illness can select people into harmful conditions that maintain mental illness. Once the loop begins, asking which factor was “the real cause” resembles asking which blade of the scissors cut the paper.

The second alternative is confounding. Childhood temperament, parental illness, genetic liability, neighborhood sorting, substance use, physical disease, and prenatal exposures can generate both adverse social conditions and psychiatric outcomes. A parent’s depression can reduce

family income and transmit genetic risk. An impulsive adolescent may be more likely to experience exclusion and later substance problems. A city may attract young adults who already have different risks than rural residents. Statistical adjustment helps only when the confounder is measured well. Controlling for a crude variable can produce an appearance of rigor while leaving the mechanism untouched.

The third alternative is measurement. Better-resourced societies may report more disorder because they recognize it. Marginalized groups may report less because disclosure is dangerous. Diagnostic practices can vary across clinicians and cultures. Social change can alter the language through which suffering becomes visible. Rising diagnosis can mean rising incidence, rising awareness, broadened criteria, increased help-seeking, administrative incentives, or all of them. The claim that “society is sicker than ever” is therefore much harder to establish than a trend line of prescriptions or survey symptoms suggests.

Strong studies attack these alternatives rather than merely acknowledging them. Longitudinal designs ask whether exposure precedes outcome. Fixed-effects models compare people with themselves across changing circumstances. Sibling and twin designs partially control shared family and genetic factors. Instrumental variables exploit a cause of exposure that should not directly affect the outcome. Difference-in-differences studies compare changes in a group affected by a policy with changes in a plausible control group. Regression discontinuity uses thresholds that assign different treatment near an arbitrary cutoff. Natural experiments study shocks such as plant closures, lotteries, benefit reforms, or disasters. Randomized controlled trials deliberately change income, housing, care, or social environment.

Each method purchases leverage by accepting a different limitation. Randomization can establish the effect of an offer without establishing what part of the offer worked. Natural experiments are rarely as clean as their name. Policy evaluations depend on assumptions about parallel trends and unaffected comparison groups. Genetically informed studies

can overcontrol pathways through which social causes operate. No design can randomize racism, war, childhood abuse, or decades of poverty ethically. The evidential task is triangular: different imperfect views should point toward the same structure.

This is why the most persuasive social evidence does not come from one dramatic correlation. It comes from convergence. Unemployment is associated with distress cross-sectionally; distress rises after job loss longitudinally; plant closures produce mental-health changes; re-employment often improves symptoms; and the association is weaker where income protection is stronger. Poverty is associated with depression; income shocks alter symptoms in some natural experiments; cash-transfer trials produce small average benefits; and mental illness also reduces income. Violence predicts post-traumatic symptoms; dose and proximity matter; quasi-experimental studies of maltreatment still find effects after addressing confounding; and interventions that provide safety reduce risk. The causal claim becomes credible because its rivals cannot explain the entire pattern.

A system need not intend harm to be causal. Markets do not have emotions, bureaucracies do not need hatred, and algorithms do not need consciousness. Causation is indifferent to motive. A scheduling system that repeatedly destroys sleep may contribute to mania or depression even if it was optimized only for labor efficiency. A benefits system that imposes uncertainty may increase distress even if every rule passed procedural review. A platform that rewards outrage can alter attention and social comparison without its designers wishing anyone ill. Social pathology often resides not in a villain but in an objective function.

The harder moral question follows. When does a causal contribution become responsibility? Science cannot settle that alone. A rainstorm may trigger a breakdown in a badly designed roof; responsibility belongs differently to weather and builder. Genetic vulnerability may make one person collapse under a demand others tolerate; that does not automatically excuse the demand. We judge systems partly by

foreseeability, avoidability, distribution, and whether those exposed can exit. A society cannot promise that no one will suffer. It can be asked why the same people repeatedly pay the psychiatric cost of its arrangements.

The conclusion of this section is modest in form and radical in consequence. “Society causes mental illness” is too vague to be scientifically true. “Social systems participate in causal pathways that generate, precipitate, define, and maintain mental disorders” is supported by substantial evidence. The second sentence sounds less revolutionary. It is more demanding. It turns the question from metaphysics into engineering: which pathways, for whom, under what conditions, and which institutional change would interrupt them?

How Society Gets Under the Skin

The phrase “social cause” can create the false image of an external force striking an otherwise sealed brain. There is no such boundary. A social event reaches the nervous system through perception, expectation, endocrine activity, immune signaling, sleep, nutrition, behavior, and the actions of other people. The route may be symbolic, but the consequence is physical. A humiliating dismissal is made of words and status; it can still alter heart rate, cortisol, sleep, appetite, rumination, alcohol use, and the probability of a depressive episode.

Stress is the most familiar bridge and the most easily abused. In ordinary language it can mean anything unpleasant. In physiology it refers to coordinated systems that mobilize an organism to meet demand. Acute stress is not necessarily harmful. It sharpens attention, releases energy, and prepares action. The problem is chronic activation without recovery, repeated unpredictability, or a demand that cannot be escaped or controlled. Bruce McEwen called the cumulative cost of adaptation *allostatic load*. The body preserves short-term function by spending long-term reserves.

Social systems are unusually capable of producing this pattern because they can make threat durable and ambiguous. A predator eventually leaves. An eviction notice continues to exist while the tenant sleeps. Debt is represented in databases that wake again each morning. Precarious work converts the future into a sequence of conditional permissions. Discrimination requires vigilance because the next injury cannot be located in advance. Bureaucratic uncertainty is powerful precisely because no single moment is catastrophic enough to justify collapse, while no moment is safe enough to permit recovery.

Control is one of the most consistent modifiers. In classic animal research, uncontrollable aversive events produced passivity and altered later learning, giving rise to the concept of learned helplessness. Human life is more complex, but perceived control repeatedly predicts the psychiatric impact of stress. The Whitehall studies of British civil servants undermined the comforting belief that occupational stress belongs mainly to high-status executives. Lower employment grades, characterized by less control and fewer resources, showed worse health. Later research on job strain found that high demands combined with low control predict depression, although effect sizes shrink when unpublished evidence and measurement limitations are included. The important variable is not simply how hard one works. It is whether effort occurs inside a credible relationship between action, reward, and exit.

The same logic appears in trauma. Post-traumatic stress is not determined only by physical magnitude. Interpersonal violence, torture, captivity, betrayal, and repeated exposure often have stronger effects than impersonal events because they reorganize assumptions about agency and human intention. A survivor is not merely remembering danger. The mind has learned a model of the world in which danger is socially generated, concealed, and likely to return. Hypervigilance is costly, but in the original environment it may have been rational.

Development makes these processes more consequential. Early life is not simply a smaller version of adulthood. The brain is building

expectations about whether signals bring care, whether other people are predictable, and whether exploration is safe. Attachment research does not imply that one parenting style mechanically determines destiny. It shows that repeated interactions become regulatory infrastructure. A responsive caregiver helps an infant manage arousal before the child can do so alone. Severe neglect removes not just affection but a biological service.

The evidence on childhood maltreatment was long dominated by observational studies, leaving open the possibility that family and genetic confounding explained part of the association with later mental illness. A meta-analysis by Baldwin and colleagues (2023) focused on quasi-experimental designs, including twin, sibling, adoption, and other approaches intended to strengthen causal inference. The association between maltreatment and mental-health problems became smaller after these controls but did not disappear. The adjusted effect was modest rather than mythical. That is scientifically important. It suggests both that conventional studies overstate causality and that severe adverse experience contributes causally to later disorder.

Small average effects can have large population consequences when exposures are common, but they also warn against deterministic storytelling. Most maltreated children do not develop the same disorder, and many do not develop a diagnosable disorder at all. Resilience is not evidence that the exposure was harmless. It is evidence that causal effects are conditional on timing, duration, biology, relationships, later opportunities, and chance. A bridge can be badly engineered even when most vehicles cross it.

Immune and inflammatory pathways offer another bridge, though public discussion often outruns the evidence. Chronic social stress is associated with inflammatory markers, and inflammation can influence mood, motivation, sleep, and cognition. Some depressed patients show elevated inflammation, but depression is heterogeneous and no single inflammatory signature defines it. Social isolation, obesity, infection, smoking, medication, poverty, and sleep all interact. The scientifically

responsible claim is not that society “inflames the brain” in one uniform process. It is that social conditions can alter immune regulation and that this may be one pathway linking adversity to symptoms in subsets of people.

Sleep is more immediate and often neglected. Shift work, overcrowded housing, neighborhood noise, caregiving burden, platform use, fear, and financial rumination all disrupt sleep. Sleep disturbance is both a symptom and a causal risk factor. It can worsen anxiety and depression; in vulnerable individuals, severe sleep loss can precipitate mania or psychotic experiences. A labor system that normalizes unstable schedules or a digital system that monetizes night-time attention can therefore participate in psychiatric risk without ever touching diagnostic categories.

Meaning is also a mechanism. Humans do not respond to events only through their sensory properties. Losing a job can mean temporary inconvenience, moral failure, liberation, social expulsion, or danger to one’s children. The same financial loss has different effects depending on welfare protection, cultural expectations, household resources, and prior experience. Social systems supply the interpretive frame. A society that treats unemployment as evidence of personal worthlessness adds shame to lost income. A culture that recognizes trauma may make symptoms legible and permit help; it may also provide a template through which distress is organized and remembered. Meaning can protect, injure, and classify.

Social defeat has been proposed as a pathway to psychosis, especially in research on migration, minority status, urbanicity, and exclusion. The hypothesis is that repeated experiences of outsider status and subordination sensitize dopaminergic or threat-related systems in vulnerable people. It is plausible and supported indirectly, but it is not a complete established mechanism. Migrant psychosis risk varies greatly by group and destination. Discrimination, ethnic density, trauma, socioeconomic conditions, diagnostic bias, cannabis exposure, and selection all matter. “Social defeat” is useful when it organizes testable

pathways; it becomes empty when it merely renames every adversity after the outcome is known.

Genes enter every part of this story. Large genomic studies now show extensive polygenic and cross-disorder architecture across psychiatric conditions (Grotzinger et al., 2026). Schizophrenia and bipolar disorder are highly heritable at the population level, major depression moderately so, and many genetic influences are shared. Rare variants can substantially increase risk. These findings refute any theory in which social systems manufacture severe mental illness from psychologically identical persons.

They do not refute social causation. Heritability describes variation within a population under its existing environments. If every person is exposed to a similar social condition, that condition contributes little to measured variation even if it is necessary for the outcome. Heritability can change when environments change. Genes may influence exposure through temperament or behavior, influence sensitivity to exposure, or influence the probability that a social response becomes persistent. A genetic liability for psychosis may remain below threshold in one environment and become clinically expressed in another. The environment is not a stage on which genes perform a fixed script; it is part of the regulatory system.

Gene-environment interaction is often invoked more confidently than it is demonstrated. Candidate-gene studies produced many celebrated findings that failed to replicate. Modern polygenic methods are more credible but still face population stratification, measurement, and causal interpretation problems. The best conclusion is broad rather than personalized: inherited liability is real, environmental effects are real, and their interaction is likely but difficult to estimate precisely. Current science cannot usually tell an individual what percentage of their illness came from society and what percentage came from genes. Causes do not arrive with invoices.

The brain itself can mislead public reasoning. Neuroimaging studies show group differences associated with trauma, poverty, institutional

deprivation, depression, and psychosis. Such findings demonstrate biological embodiment but rarely identify direction or specificity. A brain image after years of illness reflects cause, consequence, adaptation, medication, behavior, and measurement noise. Colored regions are not causal arrows. The strongest developmental studies combine imaging with prospective or experimental designs, as in the Romanian foster-care research. Even then, a neural difference is not automatically damage; it may be an adaptation with costs in another environment.

Feedback is the central architecture. A person exposed to chronic threat sleeps poorly. Poor sleep increases irritability and suspiciousness. Conflict follows. Social support retreats. Work performance declines. Income falls. Treatment becomes harder to obtain. The person receives a diagnosis that may provide care but also trigger stigma. Anticipated rejection encourages secrecy and withdrawal. Isolation amplifies symptoms. Each step is partly internal and partly external. By the time the loop stabilizes, there is no meaningful point at which society ends and illness begins.

This helps explain why treating symptoms can be necessary without being sufficient. Medication may reduce hallucinations while homelessness continues to produce vigilance. Psychotherapy may reduce self-blame while an abusive partner remains. Housing may provide safety while severe depression persists and requires treatment. Social intervention and clinical care address different links in the same network. The insistence that one must replace the other is not radical. It is mechanistically naive.

A social system becomes psychiatric infrastructure when it repeatedly controls the variables through which minds regulate themselves: predictability, agency, sleep, attachment, safety, status, belonging, and access to recovery. Some systems supply these goods. Others extract them. The science of social causation is ultimately the study of how institutions are translated into nervous systems, and how nervous systems return the translation as behavior, symptoms, and social fate.

Worlds That Enter the Child

The Children Who Learned the World Was Empty

The Romanian orphanages did not begin as a psychiatric experiment. They began as policy.

In 1966, Nicolae Ceaușescu's regime issued Decree 770, sharply restricting abortion and contraception in an effort to increase the population. Births rose, but the state did not create the material conditions required to support every family. Poverty, disability, stigma, coercive reproductive policy, and the promise that institutions could assume care combined to send large numbers of children into state facilities. By the end of the regime, the institutions had become an administrative solution to problems the state had helped create.

This history matters because the deprivation was not a private accident repeated thousands of times. It was organized. Staffing ratios, routines, physical spaces, and bureaucratic incentives produced a predictable ecology: too little individualized care, frequent changes of caregiver, minimal stimulation, and long periods in which no adult responded contingently to a child's signals. The institution did not need a policy titled "impaired attachment." It only needed to optimize custody without providing relationships.

After 1989, images of children in bare rooms circulated internationally. The moral response was immediate, but the scientific interpretation remained contested. Some children had been placed because of pre-existing disability or illness. Records were incomplete. Malnutrition and infection

were common. International adopters were unusually motivated and well resourced. If children improved after adoption, perhaps selection rather than social care explained the difference.

Two major research programs provided unusually strong evidence. The English and Romanian Adoptees study (Sonuga-Barke et al., 2017) followed children adopted into the United Kingdom, comparing those exposed to different durations of Romanian institutional deprivation with adoptees who had not experienced comparable privation. The Bucharest Early Intervention Project, led by Charles Nelson, Nathan Fox, Charles Zeanah, and Romanian colleagues and summarized in King et al. (2023), enrolled young children living in institutions and created a high-quality foster-care network. Eligible children were randomly assigned to foster care or care as usual, while never-institutionalized children formed a comparison group.

Randomizing children in adversity is ethically difficult. The researchers did not place children in institutions; all were already there. At the time, foster care was not an established governmental alternative in Bucharest, and authorities retained decisions about placement. The trial's ethical defense rested on equipoise about the effects and on building an intervention that otherwise did not exist. Critics have nevertheless asked whether researchers should have removed every child rather than randomize. The dispute is important because even the strongest social science can be produced inside unequal power. A causal design does not dissolve moral responsibility.

The findings accumulated across years. Children assigned to foster care developed more secure attachment and better cognitive outcomes than those who remained in institutional care, with earlier placement generally associated with larger benefits. Electroencephalographic measures suggested that the radically altered pattern of brain activity seen among institutionalized children became more typical after earlier foster placement. Later work reported effects on psychiatric symptoms, social functioning, stress regulation, physical growth, white-matter organization,

and cortical structure. Not every outcome changed, and some differences persisted into adolescence and adulthood. The intervention was not a reset button.

The most scientifically revealing result was heterogeneity. Severe deprivation did not produce a single “institutional syndrome” in every child. Some showed relatively broad recovery. Others retained attention problems, social disinhibition, autistic-like features associated with deprivation, cognitive difficulties, or emotional symptoms. Timing mattered, but not as a magical deadline. The quality and stability of later care mattered. Biological differences likely influenced susceptibility. The institution changed probability distributions rather than stamping identical injuries.

This is what social creation usually looks like. It is not the insertion of a discrete entity called mental illness. It is the systematic alteration of developmental inputs from which later capacities are built. A child deprived of stable response does not merely feel lonely. The child receives less opportunity to learn that distress can be co-regulated, that faces predict particular people, that signals produce proportionate reactions, and that exploration can occur against a secure background. Social absence becomes developmental information.

The research also distinguishes deprivation from threat. Abuse exposes a child to harmful stimulation; institutional neglect removes expected stimulation and contingent care. Both can produce psychopathology, but through partly different pathways. Developmental neuroscience increasingly separates dimensions such as threat, deprivation, unpredictability, and loss rather than treating all adverse childhood experiences as interchangeable points on a score. A household with little money but stable affection is not equivalent to a violent household. An institution with adequate calories but no enduring caregiver is not adequate care.

This precision corrects a popular misuse of the adverse childhood experiences literature. The original ACE studies revealed a graded

association between accumulated adversity and adult health problems. Public communication often turned the score into a personal destiny index. Yet ACE categories differ in severity, timing, duration, and mechanism; retrospective reports have limitations; and cumulative scores can conceal protective relationships. A high score is neither a diagnosis nor a forecast. The quasi-experimental meta-analysis by Baldwin and colleagues (2023) found that stronger controls reduced the estimated effect of maltreatment on mental-health problems. That reduction is not an embarrassment. It is the difference between advocacy and causal science.

The remaining effect was still meaningful. It implies that prevention of maltreatment and severe neglect would prevent some later mental illness, even though it would not prevent all of it. It also implies that people exposed to adversity should not be told that every future difficulty was prewritten in childhood. Determinism can become a second injury, especially when trauma language is used to convert a probabilistic risk into an identity.

A 2024 analysis in *JAMA Psychiatry* translated those quasi-experimental estimates into population terms. Lucinda Grummitt and colleagues estimated that childhood maltreatment accounted for about one fifth of depression and roughly two fifths of suicide attempts in Australia (Grummitt et al., 2024). These are modeled population-attributable fractions, not outcomes observed after maltreatment was somehow abolished. They depend on assumptions about causality, measurement, and whether pooled effects transport to the Australian population. Their importance is therefore not the apparent precision of the percentages. It is the demonstration that a modest individual-level causal effect can become a large public burden when exposure is common.

History supplies earlier versions of the same lesson. Foundling hospitals in eighteenth- and nineteenth-century Europe often had catastrophic mortality, produced by disease, malnutrition, crowding, and discontinuity of care. Twentieth-century research by René Spitz described infants in

institutions who became withdrawn and developmentally impaired, though his methods and terminology belong to an earlier scientific era. John Bowlby's work on maternal care was influential but sometimes overgeneralized into a prescription that only one family form could support healthy attachment. The durable insight is narrower: young children require stable, responsive relationships. The identity and gender of the caregiver are not the causal essence; contingency and continuity are.

A social system can also harm children through the conditions imposed on caregivers. Parental leave, work schedules, housing, health care, income, community violence, and immigration policy all shape the capacity to provide responsive care. It is politically convenient to locate every developmental failure in parental character. Families matter enormously, but families are not outside political economy. A caregiver working two unstable jobs does not possess the same temporal resources as one with paid leave and secure housing. This does not make neglect inevitable or excuse abuse. It identifies upstream variables that alter risk.

The Cherokee income experiment makes the point from another direction. In the Great Smoky Mountains Study, researchers were following children in western North Carolina when a casino opened on the Eastern Band of Cherokee reservation. Tribal members began receiving annual income supplements. Families that moved out of poverty after the payments saw children's psychiatric symptoms, especially conduct and oppositional symptoms, decline toward the level of children who had never been poor. Because the researchers had data before and after an externally generated income change, the study offered stronger evidence than a conventional poverty comparison. Later follow-up linked the supplements to some improved adult outcomes.

The casino did not change genes, erase every family problem, or supply psychotherapy. It altered material pressure. Analyses suggested that improved parental supervision and family relations were part of the pathway. Money was translated into time, conflict, predictability, and care. The result does not prove that cash cures childhood mental illness. It

demonstrates that what appears as a child's behavioral disorder can be partly responsive to a change in family economic conditions.

This finding should not be romanticized. Income supplements can produce different effects depending on size, regularity, household control, local prices, violence, substance markets, and services. Cash-transfer reviews generally find small positive average effects on mental health, not miracles. Some outcomes do not change. Yet a small causal effect is conceptually large when it contradicts the assumption that treatment-relevant variables reside only in clinics.

Institutional deprivation and family poverty lie on a continuum only in the abstract. The Romanian institutions represented extreme, relationally barren environments; most low-income families provide abundant love and stimulation under difficult conditions. Conflating poverty with neglect is both scientifically false and socially punitive. The common mechanism is not "poor parenting." It is the degree to which systems supply or obstruct the conditions of development.

The deepest lesson from Bucharest is therefore not that institutions are bad and families good. Families can be terrifying, and foster systems can be unstable. Small institutions can reproduce abandonment; large organizations can sometimes create continuity. The causal variable is the lived pattern of response. A humane system must be designed around the developmental fact that care is not a service delivered to a child in units. It is a relationship that becomes part of the child's regulatory machinery.

When the children stopped crying, they had not adapted to nothing. They had adapted to an institution. Their silence was learned evidence about the world. The later studies showed that a different world could modify that evidence, especially when it arrived early and remained stable. A society had entered the mind by failing to answer. Another social arrangement entered by answering back.

The Manufacture of Threat

In the first months of the First World War, physicians expected modern soldiers to suffer modern wounds. They did not expect men who had no visible injury to become blind, mute, paralyzed, sleepless, confused, or seized by tremor. The term “shell shock” suggested that artillery blast had physically damaged the nervous system. Some cases fit that possibility. Others had never been near an explosion. As the war continued, the diagnosis became a battlefield on which medicine, military discipline, masculinity, compensation, and national need fought for control.

A soldier’s symptoms could be interpreted as neurological injury, emotional collapse, cowardice, malingering, or an understandable response to industrialized slaughter. Treatment ranged from rest and psychological support to punishment and electric shocks. The category changed because the institution needed two incompatible things: recognition that men had been injured and a mechanism for returning them to combat. The history of shell shock is not a primitive preface to the discovery of post-traumatic stress disorder. It shows that the naming and treatment of trauma are shaped by the organization that produced the exposure.

The later history of PTSD followed a similar pattern. Survivors of war, sexual violence, genocide, accidents, and disaster had long described recognizable symptoms, but the diagnosis entered the DSM in 1980 after scientific work and political advocacy, especially by Vietnam veterans and clinicians. The category made an important causal statement: an event outside the person could be etiologically central. Earlier psychiatric language often emphasized constitutional weakness or neurosis. PTSD placed the stressor in the criteria.

That achievement created new difficulties. Trauma became a portable explanation for diverse suffering. Definitions of qualifying events expanded and contracted. Cultural expressions varied. Researchers debated delayed onset, complex trauma, moral injury, and the boundary between normal post-traumatic distress and disorder. The diagnostic history illustrates the

double nature of classification. A category can correct a moral error by locating causation in violence, while still requiring scientific discipline about who develops persistent illness and why.

The epidemiological evidence that organized violence produces mental disorder is among the strongest in social psychiatry. Steel and colleagues (2009) synthesized 181 surveys from conflict-affected and displaced populations. After accounting for methodological differences, torture and cumulative exposure were strongly associated with PTSD and depression. Charlson and colleagues (2019) later estimated that roughly one in five people in conflict settings had depression, anxiety, PTSD, bipolar disorder, or schizophrenia at a point in time, although estimates vary by setting and diagnostic method. The precise prevalence is contestable. The excess burden is not.

War is a social system at its most explicit: institutions organize exposure to killing, loss, displacement, uncertainty, and moral violation. Yet even here, the outcome is not uniform. Most people exposed to trauma do not develop chronic PTSD. Some show transient symptoms, some remain resilient, and some experience post-traumatic growth alongside pain. Prior vulnerability, dose, proximity, injury, bereavement, social support, meaning, and post-conflict conditions all matter. The existence of resilience does not disprove causal harm. It prevents the causal story from becoming a prophecy.

Post-migration conditions reveal how systems maintain what violence initiates. Refugees may escape the original threat but encounter detention, family separation, insecure legal status, unemployment, poor housing, discrimination, and restricted access to care. Meta-analytic work has repeatedly linked post-migration stressors to depression, anxiety, and PTSD. A person can be physically outside the war and socially unable to arrive anywhere else. The asylum bureaucracy becomes an extension of uncertainty.

Torture makes the role of social intention especially clear. Its purpose is not only to inflict pain but to reorganize the victim's relationship to

agency, trust, body, and community. The torturer uses an institution's authority to demonstrate that ordinary rules no longer protect the person. This is why legal recognition, testimony, social restoration, and safety can matter alongside clinical treatment. The injury includes a political proposition about who may be harmed without consequence.

Intimate-partner violence reproduces similar dynamics at household scale. Reviews, including the Lancet Psychiatry Commission led by Sian Oram and later umbrella analyses, show robust associations between partner violence and depression, PTSD, anxiety, substance use, and suicidality. Bidirectionality and confounding matter: pre-existing mental illness can increase vulnerability to victimization, and substance problems can participate in violent relationships. Longitudinal evidence nevertheless supports violence as a cause of later mental-health problems. Coercive control is particularly instructive because it can be psychologically destructive without constant physical assault. Surveillance, isolation, financial restriction, threats, and unpredictable punishment convert daily life into a controlled environment.

Clinical advice that ignores the environment can become absurd. Cognitive restructuring cannot make a credible threat irrational. Relaxation does not restore access to money. Medication may help a survivor sleep, but sedation can be dangerous when vigilance remains necessary. Effective care often requires safety planning, legal assistance, housing, income, and protection. The social intervention is not an optional political supplement to the real treatment. It changes the causal exposure.

Community violence affects children even when they are not direct victims. Patrick Fowler and colleagues' meta-analysis (2009) found associations between exposure and post-traumatic, internalizing, and externalizing symptoms, with stronger effects for direct victimization. Repeated gunfire, neighborhood loss, policing, and the expectation of violence shape movement, sleep, concentration, and trust. Schools may then interpret hypervigilance as defiance, dissociation as inattention, and

defensive aggression as character. A symptom formed in one system is punished in another.

This does not mean every disruptive behavior is trauma or that accountability becomes impossible. Trauma-informed rhetoric can become so expansive that it explains everything after the fact. The more useful question is functional: what did the behavior protect against, what maintains it now, and what response reduces future harm? Understanding is not acquittal. It is a prerequisite for effective design.

Racism adds a distributed form of threat. Paradies and colleagues (2015) synthesized hundreds of studies and found a consistent association between reported racism and poorer mental health, strongest for depression, anxiety, and distress. Most studies were observational and relied on self-report, so they cannot establish every causal step. Yet the broader evidence includes longitudinal studies, audit studies of discrimination, policy comparisons, physiological research, and quasi-experimental work on structural stigma. The convergence makes it implausible that the entire association is produced by reporting style.

The psychiatric system can intensify racial harm. Historical diagnoses such as drapetomania, proposed in the American slaveholding South to pathologize enslaved people who fled captivity, reveal medicine serving domination in almost pure form. The term was never a legitimate scientific discovery. It was a moral defense of slavery translated into disease language. Later American psychiatry repeatedly racialized danger, aggression, and schizophrenia, while underrecognizing mood disorders or trauma in Black patients. Contemporary disparities do not prove that every diagnosis is biased, but they make diagnostic neutrality an empirical question rather than a professional entitlement.

Structural stigma research provides an unusually direct link between policy and mental health. Mark Hatzenbuehler and colleagues examined sexual-minority populations across different legal and attitudinal environments. In one natural-experimental line of work, lesbian, gay, and bisexual adults living in US states that passed constitutional bans on

same-sex marriage showed increased psychiatric morbidity relative to comparison groups, while policy protection and supportive school climates were associated with better outcomes in other studies. These designs remain vulnerable to state-level differences and migration, but they demonstrate a pathway more specific than generic “stress.” The law changes public status, expectations of safety, concealment, and daily interactions.

The history of homosexuality in psychiatry shows the reciprocal process. For much of the twentieth century, the diagnosis made social persecution appear clinically justified. The APA removed homosexuality from the DSM in 1973 after empirical criticism, internal debate, and organized activism. The change is sometimes described either as a scientific correction or as a political capitulation. It was both scientific and political in the proper senses. Research failed to demonstrate inherent pathology, while gay activists made visible the values and harms concealed by professional consensus. Politics did not corrupt a previously pure category; it helped psychiatry recognize that the category had already been political.

Violence and discrimination also alter symptom content. Persecutory delusions draw on available threats. Trauma-related voices may reproduce abusers. Cultural narratives supply supernatural, technological, religious, or political forms. This does not make the experiences unreal or reducible to discourse. It shows that even severe symptoms use social material. The mind constructs terror from what a world has taught it to fear.

Online survivor discussions repeatedly return to a complaint that professionals ask, “What is wrong with you?” before asking, “What happened to you?” The reformulation is valuable, especially where violence has been ignored. It can also become another universal key. Some people have no identifiable trauma before illness. Others experience both inherited vulnerability and adversity. A better clinical sequence adds questions rather than replacing them: what happened, what changed in the body and mind, what is happening now, what protects the person, and what treatment is acceptable?

A system creates threat when it organizes exposure, denies exit, and then interprets adaptation as personal defect. It maintains illness when the threat continues after diagnosis or when the survivor must repeatedly prove the injury to institutions that control safety and recognition. War, domestic abuse, racism, and political stigma differ in scale and mechanism, but each demonstrates a fact that an individualizing model cannot absorb: sometimes the mind's alarm is not malfunctioning in isolation. It has been trained by a social world that kept ringing it.

Bullying offers a less extreme but unusually legible example. Children are not randomly assigned to be victimized. Neurodevelopmental differences, anxiety, social skills, body size, disability, family adversity, and prior symptoms can increase vulnerability. This makes causation difficult to isolate. Yet prospective studies consistently show that victimization predicts later anxiety, depression, self-harm, and other difficulties even after adjustment for prior problems. In the Great Smoky Mountains cohort, children bullied by peers had elevated rates of psychiatric problems in young adulthood; those who were both bullies and victims often fared worst (Copeland et al., 2013).

Intervention evidence strengthens the causal case. The Finnish KiVa program attempts to change the peer ecology rather than merely teaching the targeted child to cope. It focuses on the bystanders whose laughter, silence, and status calculations make bullying rewarding. Trials have found reductions in victimization and, in some studies, improvements in anxiety, depression, and peer perceptions (Williford et al., 2012). A large British evaluation published in 2024 found modest reductions in victimization but no clear broad improvement in child mental-health outcomes over the study period, a result that is scientifically valuable precisely because it limits the story (Bowes et al., 2024). Interrupting one pathway does not guarantee rapid change in every symptom, and imported programs do not reproduce identical effects in every institutional culture.

Bullying demonstrates how a social system can exist at the scale of a classroom. No law orders children to terrorize a classmate. A stable pattern

emerges from incentives: aggressors gain status, audiences receive entertainment or safety, teachers miss dispersed incidents, and victims become risky allies. The school can sustain the disease process while sincerely offering counseling to the child it fails to protect. Individual therapy may help, but it should not become the institution's method of adapting the victim to victimization.

The language of “the body keeping score” is compelling but can imply a perfect ledger in which every injustice receives a biological entry. The body is less orderly. It compensates, forgets, adapts, misfires, and changes. Some people show psychiatric symptoms without conventional stress biomarkers; others carry high physiological burden while reporting well-being. Biological measures are not moral truth detectors. They help specify pathways, not certify that suffering deserves recognition.

Public internet narratives often collapse these distinctions because biography is experienced as a whole. People write that an abusive family “gave” them borderline personality disorder, that school bullying “caused” social anxiety, or that war “made” them hypervigilant. Others resist causal language because siblings exposed to the same home developed differently. Both observations can be true. Shared environments are not identical environments, siblings differ genetically, roles within families diverge, and chance events accumulate. A causal exposure need not produce a uniform outcome.

There is also a political danger in locating every adult problem in childhood. The trauma economy can turn a complex life into an excavation project: symptoms are assumed to conceal an early wound, and recovery depends on finding it. This can create false certainty, neglect present conditions, and encourage people to interpret ordinary memory gaps as evidence of hidden abuse. Science supports strong average associations between childhood adversity and later disorder. It does not authorize a clinician, influencer, or algorithm to infer a particular trauma from a symptom.

The Political Economy of Distress

Money as a Psychiatric Variable

Money is not an antidepressant molecule. It has no receptor, no therapeutic blood level, and no standard dose. Yet money controls many of the variables through which mental health is protected: food, housing, sleep, transport, time, privacy, care, status, and the ability to leave danger. To call poverty a social determinant can sound bloodless. In lived experience, poverty is an environment that follows the person indoors.

The association between poverty and common mental disorders is among the most consistent in psychiatric epidemiology. Poorer people report more depression and anxiety, and people with severe mental illness are disproportionately poor. The difficulty is direction. Depression can reduce productivity, employment, education, and earnings. Psychosis can produce downward mobility. Families with inherited psychiatric liability may also face economic disadvantage. The poor are not a randomized group.

Johannes Haushofer, Vikram Patel, Gautam Rao, Frank Schilbach, Matthew Ridley, and colleagues have helped move the field from association toward causation. Ridley and co-authors (2020) reviewed evidence from natural experiments and randomized interventions in Science and concluded that the relationship is bidirectional: poverty causes some depression and anxiety, while mental illness also causes poverty. The mechanisms are not a single “poverty stress.” They include income

volatility, exposure to shocks, lower access to treatment, worse physical health, cognitive load, shame, violence, and reduced control.

A 2024 Mendelian-randomization study using UK Biobank data reached a similarly nonuniform conclusion. Mattia Marchi and colleagues found evidence consistent with bidirectional causal relations between poverty and both schizophrenia and attention-deficit/hyperactivity disorder, and with a pathway from poverty toward major depression, while other diagnoses showed different patterns (Marchi et al., 2024). Mendelian randomization does not literally randomize people into poverty. It uses genetic variants as instruments and depends on demanding assumptions about pleiotropy, ancestry, and how well the instruments represent the exposure. Its value here is triangulation: a method with weaknesses unlike conventional observation again suggested reciprocity and diagnostic specificity rather than one universal poverty effect.

The phrase cognitive load is useful when it does not become a new way of blaming the poor. Scarcity requires attention. A person deciding which bill can be delayed is performing difficult optimization under penalty. That attention is unavailable for long-term planning, sleep, parenting, or treatment adherence. Laboratory and field studies suggest that financial concerns can temporarily consume cognitive bandwidth, though effect sizes and replications vary. The robust insight is simpler: uncertainty imposes work, and the work is rarely visible to institutions that demand punctual forms, stable schedules, and perfect foresight.

Debt adds a future-oriented form of threat. In a meta-analysis, Elizabeth Richardson and colleagues (2013) found that unsecured debt was associated with substantially higher odds of mental disorder, depression, substance problems, and suicidal outcomes. Most included studies were observational, and illness can produce debt. Yet longitudinal evidence and reports of debt-collection pressure, legal threat, and loss of control support a reciprocal pathway. Debt is not only low income. It is income already claimed by the past.

Unemployment supplies stronger temporal evidence. Karsten Paul and Klaus Moser's meta-analysis (2009) combined cross-sectional, longitudinal, and intervention-related findings. Unemployed people had markedly worse mental health than employed people, and symptoms often rose after job loss and improved after re-employment. The effect was larger where unemployment protection was weaker and where work held greater cultural centrality. A job provides income, but also time structure, social contact, identity, and recognized contribution. Losing it can remove several regulatory systems at once.

Again, selection matters. People with poor mental health are more likely to lose jobs and less likely to be rehired. Recent administrative and quasi-experimental studies continue to find effects in both directions. This reciprocal evidence should change policy design. Employment services that punish inconsistent participation may interpret symptoms as noncompliance, remove income, and thereby intensify the condition that obstructs re-employment. A system designed to test willingness can become a machine for producing incapacity.

Economic shocks reveal the population scale. Reviews of recessions find increases in distress, depression, suicide, and service use, especially among people who lose work, housing, or income. The effects are not inevitable. Countries with stronger social protection often show smaller increases. Research associated with David Stuckler and colleagues argued that active labor-market programs and generous protections can buffer suicide during downturns, although ecological comparisons cannot fully isolate policy effects. The key observation is comparative: the same macroeconomic contraction does not produce the same psychiatric outcome under every welfare regime.

Austerity policies can compound recession by reducing services when need rises. Studies of Greece after the financial crisis reported increased suicides and mental-health deterioration following severe austerity, but interrupted time-series and ecological designs cannot assign every change to a single policy. Migration, reporting, prior trends, and simultaneous shocks

complicate interpretation. The evidence is strongest not as a verdict on one government but as a warning that fiscal choices distribute psychological exposure.

Welfare reform offers more precise quasi-experiments. The rollout of Universal Credit in the United Kingdom was associated (Reeves et al., 2020) with increased psychological distress among unemployed recipients compared with groups not yet exposed. Researchers could exploit the staggered introduction, but the design still depended on assumptions about comparable trends. The result is plausible because the reform involved waiting periods, conditionality, uncertainty, and financial strain. A benefit can protect mental health through income and harm it through administration. The amount transferred and the manner of transfer are both exposures.

The Finnish basic-income experiment tested a different administrative logic. Two thousand unemployed people were randomly assigned to receive a regular, unconditional monthly payment for two years, without losing it when they took work. Employment effects were modest, but recipients reported better well-being, less mental strain, and greater confidence in the future than controls. Self-reported outcomes and the specific Finnish context limit generalization. Still, the experiment suggests that predictability and reduced conditionality may carry psychological value even when labor-market behavior changes little.

Cash-transfer research in low- and middle-income countries reaches a similarly measured conclusion. Systematic reviews and meta-analyses generally find small improvements in depression, anxiety, or psychological well-being. Some programs show no effect, and effects can fade. Transfers differ in size, frequency, recipient, conditions, and local economy. The pooled result is not “money cures depression.” It is that economic intervention moves mental-health outcomes in the expected direction often enough to reject the belief that poverty-related distress is merely a fixed trait of poor people.

The Great Smoky Mountains natural experiment (Costello et al., 2003) remains unusually vivid because children were already being followed when casino revenue began reaching Cherokee families. The reduction in some child psychiatric symptoms after families escaped poverty showed that a behavioral phenotype could respond to a material change. Yet the strongest effects were not uniform across diagnoses. Anxiety and depression did not simply vanish. This is typical. Social interventions often improve the outcomes closest to the mechanism and leave other pathways untouched.

Minimum-wage studies illustrate why policy science requires restraint. A quasi-natural experiment around the introduction of the British national minimum wage found (Reeves et al., 2017) reduced depressive symptoms among low-paid workers whose wages increased. Studies in the United States have linked higher minimum wages to lower suicide rates among less-educated adults, especially during high unemployment. Other analyses find small, null, or mixed health effects, and a recent large study found no compelling improvement across multiple child and adolescent mental-health outcomes. Employment effects, price changes, eligibility, household composition, and enforcement all modify exposure. A policy can be beneficial without every study becoming positive.

These mixed findings are not a reason to retreat to clinical individualism. They show that “income” is not one intervention. A small wage rise for a worker who loses hours differs from an unconditional transfer. A payment that stabilizes rent differs from one absorbed by medical debt. Money’s psychiatric effect depends on what it changes in the person’s feasible world.

Public forums describe this world with less methodological caution and more phenomenological accuracy. People write that therapy cannot negotiate their rent, that a job they hate protects them from the terror of losing insurance, or that every coping strategy fails under constant survival pressure. Others answer that they are financially secure and still severely depressed. Both testimonies are compatible with the evidence. Poverty is

neither necessary nor sufficient for mental illness. It is a causal risk and maintenance factor whose removal helps some people substantially, many people modestly, and others not at all.

The counterexample of affluent suffering is often presented as if it refuted social causation. It refutes only exclusivity. Wealth can coexist with abuse, loneliness, status anxiety, purposelessness, discrimination, genetic liability, sleep loss, medical disease, or no identifiable precipitant. Social systems operate through more than money, and biology does not ask for a bank statement.

The opposite error is to treat medication or psychotherapy as instruments that return people to exploitative conditions. Sometimes they are used that way. More often, patients seek relief because suffering itself has become an oppressor. A person may need an antidepressant and a debt moratorium, trauma therapy and safe housing, sleep treatment and a less destructive shift. The moral priority should not be choosing the purest explanation. It should be expanding the set of effective exits.

Money becomes a psychiatric variable when institutions use it to control safety, time, and possibility. The causal evidence is strongest for modest average effects, important subgroup effects, and reciprocal loops. That conclusion lacks the elegance of “poverty causes depression” and the comfort of “depression is a brain disease.” It is closer to reality. Economic systems do not write the whole diagnosis. They often decide how much room a person has to recover from it.

That final result matters because it reveals an interaction between an event and a system. Unemployment is not a fixed dose. It can mean a protected interval with income replacement, retraining, health coverage, and realistic re-entry. It can mean immediate financial crisis, shame, loss of medical care, and suspicion from a punitive benefits bureaucracy. The labor-market event is similar; the psychiatric exposure is not.

Income is not simply a proxy for character or consumption. It purchases control over exposure. Money can move a family away from

violence, buy time, secure childcare, replace a broken appliance before it becomes a crisis, and make refusal possible. It also changes how institutions treat a person. The affluent customer negotiates; the poor claimant proves eligibility. The same need is processed as preference at one income and dependency at another.

Small effects are easily mocked. Cash does not cure schizophrenia; therefore, one might conclude, poverty was never causal. This is equivalent to observing that clean water does not cure every infection and dismissing sanitation. Population interventions often shift risk distributions rather than rescue every individual. An average effect can be modest because some recipients were not mentally ill, some problems were unrelated to income, transfers were too small or temporary, and local prices or family demands absorbed the gain. The correct question is not whether money behaves like a universal medication but which causal pressures it removes.

Debt may exert a distinct pressure because it combines scarcity with pursuit. A person without money faces limitation; a person in debt may face recurring reminders, fees, legal threats, and moral judgment. A systematic review and meta-analysis found strong associations between unsecured debt and mental and physical health, while warning that most evidence was observational (Richardson et al., 2013). A quasi-experimental debt-relief program in Singapore provided sharper evidence. Low-income participants received unexpected relief from chronic debt. Psychological functioning improved, anxiety fell, and cognitive performance and decision-making changed. The researchers found that clearing more separate debt accounts was associated with larger benefits even when the total amount relieved was similar, suggesting that the mental burden of fragmented obligations matters (Ong et al., 2019).

Debt is thus not only a number but an attention architecture. Each account creates a deadline, threat, and unresolved task. Scarcity research has shown how urgent financial demands can capture cognitive bandwidth, although claims about a general “scarcity mindset” should not be stretched beyond specific evidence. The humane inference is simpler: a

person who appears disorganized may be managing an environment designed to interrupt them continuously.

The Architecture of Power

The modern workplace rarely resembles a psychiatric ward. That does not prevent it from regulating sleep, autonomy, status, surveillance, social contact, and exposure to humiliation. These are mental-health variables even when they appear on a management dashboard under other names.

Research on job strain has repeatedly found that high demands combined with low decision latitude predict depression. An individual-participant meta-analysis led by Ida Madsen and colleagues (2017) found an elevated risk of clinical depression among workers exposed to job strain. The estimate was stronger in published studies than in unpublished data, a reminder that publication bias can make social hazards look cleaner than they are. The association survived, but its magnitude became more modest. Science advanced by becoming less dramatic.

Workplace bullying shows a similar pattern. A meta-analysis by Verkuil and colleagues (2015) found moderate cross-sectional associations with depression, anxiety, and stress, and smaller but persistent longitudinal effects. Reverse causation is possible: distressed employees may perceive interactions differently or become targets. Yet repeated hostile behavior, power imbalance, and inability to exit form a credible causal exposure. The organizational response determines whether an interpersonal conflict becomes a system. When reporting threatens employment or managers reward the aggressor's productivity, the institution supplies maintenance.

Long working hours and unstable schedules are associated with depressive symptoms, sleep disturbance, and other health problems, though effects vary by occupation and self-selection. The most dangerous arrangement is often not intensity alone but intensity without control. A surgeon and a warehouse worker may both work at speed; only one may

have status, discretion, recovery resources, and the expectation that overload is temporary. The nervous system experiences the schedule inside a social interpretation of power.

Housing is even more fundamental because it determines whether recovery has a place to occur. Longitudinal studies link housing-payment problems, eviction, overcrowding, poor conditions, and homelessness to common mental disorders. The direction is again reciprocal. Mental illness can reduce income and disrupt tenancy; housing instability can worsen symptoms. Stronger designs have begun to separate the loop. Studies following changes in eviction status find subsequent declines in mental and self-rated health. During the COVID-19 pandemic, stronger state eviction moratoria in the United States were associated with less mental distress, functioning as a policy natural experiment. Such studies cannot eliminate every difference between states, but they show that legal protection can alter psychological outcomes before anyone becomes literally homeless.

Housing First trials provide a more direct intervention. The model offers permanent housing without requiring sobriety or treatment compliance as a precondition. Across studies, Housing First produces large improvements in housing stability. Effects on psychiatric symptoms, substance use, and general health are more mixed. The Canadian At Home/Chez Soi trial found strong housing benefits and some quality-of-life gains, but not a universal clinical transformation. This is not failure. It reveals that homelessness is one cause among several. A home removes an assault on recovery; it does not erase psychosis, addiction, trauma, or loneliness.

The result also exposes a category mistake. Housing is sometimes expected to prove its value by curing psychiatric disorder. We do not ask antibiotics to secure an apartment. A stable home is a legitimate outcome and a platform from which treatment becomes possible. Social interventions should be judged for the causal link they are designed to change.

The Moving to Opportunity experiment tested whether neighborhoods themselves matter. Low-income families in several US cities were randomly offered different forms of housing assistance, including vouchers intended to support moves to lower-poverty neighborhoods. Adults offered the opportunity later showed better mental health on some measures. Outcomes for adolescents were more complicated. Girls often benefited, while boys in some analyses showed worse behavioral or mental-health outcomes. Later economic research found substantial long-term benefits for children who moved when young, but not for those who moved later.

The heterogeneity is scientifically valuable. “A better neighborhood” is not a uniform dose. Moving can remove violence and concentrated disadvantage while disrupting friendships, kin networks, identity, and informal protection. Boys and girls may encounter new neighborhoods differently. Families may move again. A voucher changes location but not necessarily discrimination, school quality, commute, or social acceptance. The experiment refutes both neighborhood fatalism and neighborhood simplicity.

Cities present the same problem at population scale. A long research tradition finds that schizophrenia and other psychotic disorders are more common in more urban environments. Vassos and colleagues’ meta-analysis (2012) reported a dose-response pattern, with risk in the most urban settings more than twice that in the most rural. The association persists across many studies, but “urbanicity” is a container, not a mechanism. Pollution, infection, social fragmentation, inequality, housing density, crime, cannabis availability, selective migration, diagnostic access, and anonymity may all contribute.

The most plausible account is interactive. Cities concentrate opportunity and risk. They provide specialized care, diverse communities, employment, and escape from restrictive local norms. They also produce crowding, comparison, weak ties, surveillance, and unequal exposure. For some people, anonymity is freedom; for others, it is social isolation in the

presence of millions. The average association cannot tell a person whether a city is pathogenic for them.

Migration sharpens the social architecture of psychosis. Meta-analyses have repeatedly found elevated psychosis incidence among many migrant and minority groups, often around twice the majority-population rate, with large variation. Genetic selection alone is unlikely to explain the pattern, because risk changes across destinations and generations and is not uniform across migrant groups. Diagnostic bias, trauma, socioeconomic disadvantage, discrimination, and cannabis exposure remain important alternatives and mediators.

The ethnic-density effect is especially revealing. Several studies find that minority individuals living where their own ethnic group is less represented have higher psychosis risk than those in higher own-group density areas. Baker and colleagues' systematic review and meta-analysis (2021) supported this pattern, particularly for Black populations. The result is not an argument for segregation. It suggests that social isolation, discrimination, identity threat, and access to support modify risk. The same individual characteristic has different psychiatric consequences in different relational environments.

Racism operates within this architecture as repeated exposure and as allocation. Paradies and colleagues' large meta-analysis (2015) found a consistent relationship between reported racism and poorer mental health. Structural studies extend the evidence beyond perception by examining laws, neighborhood segregation, institutional practices, and cultural prejudice. No single measure captures racism, and people who report discrimination may differ in awareness or exposure. Yet it would require an implausibly elaborate confounding story to explain why discrimination, exclusion, policy stigma, and low group density repeatedly point in the same direction.

The social-defeat hypothesis attempts to connect these findings to psychosis biology. Repeated subordinate or outsider experience may sensitize threat and dopamine systems, increasing psychosis risk among

susceptible individuals. The hypothesis is compelling because it bridges social exposure and neurobiology. It remains incomplete. Dopamine dysregulation is not unique to one social experience, and social defeat is difficult to measure independently of distress. The concept should guide experiments, not substitute for them.

Work, housing, city, and migration converge on one variable that psychiatry often treats as background: feasible agency. Can the person alter the condition, obtain support, or leave without catastrophe? A high-demand job with genuine autonomy differs from algorithmic scheduling. A small apartment chosen for city life differs from overcrowding without exit. Migration by opportunity differs from flight under threat. The material exposure and its distribution of power jointly shape risk.

People in online communities often describe the diagnosis as arriving after a long chain of institutional encounters: school punishment, precarious work, lost housing, emergency care, police contact, and benefits assessment. Once diagnosed, they encounter a second chain in which the label affects tenancy, employment, credibility, and relationships. The disorder may have preceded the exclusion, but exclusion becomes part of its course. In schizophrenia forums, participants repeatedly describe being treated as dangerous, incompetent, or permanently unemployable. Such posts are not prevalence data. They are descriptions of a mechanism already documented in stigma research.

Supported employment shows that the mechanism is modifiable. Individual Placement and Support, which helps people with severe mental illness obtain competitive work rapidly and provides ongoing support, consistently outperforms traditional stepwise vocational rehabilitation on employment outcomes across countries and subgroups. Employment does not cure schizophrenia. It can restore income, routine, identity, social contact, and evidence against the expectation of permanent incapacity. The intervention works partly by refusing the institutional rule that people must first become fully well before entering ordinary life.

This is the central insight of social architecture. Systems do not merely expose people to bad events. They arrange dependencies. A job controls insurance; a landlord controls safety; a diagnosis controls access and credibility; a neighborhood controls exposure and belonging. Mental illness emerges and persists inside these dependencies. The relevant treatment may be a molecule, a relationship, a legal right, an income, a room with a lock, or a redesign of the rule that made all of them conditional.

The internet's everyday political economy appears in recurring complaints: "Therapy cannot fix my rent"; "mindfulness will not make a twelve-hour shift humane"; "my diagnosis is a normal response to capitalism." These statements contain insight and exaggeration. Therapy cannot repeal rent, but it can help someone survive, organize, negotiate, or leave. Not every depression is a labor dispute. The slogan becomes harmful when it implies that a person should refuse treatment until society is repaired. It becomes useful when it prevents treatment from becoming an alibi for avoidable conditions.

Employers and institutions increasingly offer resilience training, meditation apps, and employee assistance while preserving the source of strain. Such programs can help individuals and still operate as moral laundering. The test is not whether an intervention is psychological. It is whether the organization also examines workload, control, fairness, security, and harassment. Teaching breathing techniques in a smoke-filled room is not wrong. Refusing to open a window is.

The reverse error is to imagine that structural reform produces psychological health automatically. Generous welfare systems still contain loneliness, addiction, family violence, and severe mental illness. Benefits can be administered in demeaning ways. Jobs created by public policy can be meaningless or coercive. Income may be spent under pressure from abusive relatives. Housing can isolate. Social goods work through implementation and relationship, not merely through budget lines.

The political economy of distress is therefore not a conspiracy theory about elites manufacturing patients. It is a study of how ordinary institutions price control, security, time, and dignity. Markets are powerful coordination mechanisms, but they do not automatically charge firms for anxiety imposed on workers, landlords for instability imposed on tenants, creditors for cognitive burden, or platforms for sleep lost to engagement. Those costs appear elsewhere: in clinics, families, disability systems, emergency rooms, and shortened lives.

A mature mental-health policy would follow the costs upstream without pretending that every upstream intervention is sufficient. It would treat employment protection, wage floors, debt procedures, housing, and social insurance as part of psychiatric prevention while preserving high-quality treatment. It would ask not only whether people can adapt to the economy but what kinds of minds the economy repeatedly rewards, exhausts, excludes, and breaks.

The clinical file tells the story of a person. The economic system writes in it before the first appointment.

Thirty Ways to Organize Distress

Before the State: Small Worlds, Harsh Worlds

There is a temptation, whenever modern life feels intolerable, to imagine that the human mind was designed for a lost social paradise. The story usually begins with a small band, face-to-face relations, meaningful work, immediate contact with nature, little bureaucracy, no rent, no inbox, and no algorithmic feed. Something in this picture is almost certainly right. Human beings did evolve in small-scale societies, and many features of contemporary life are historically novel. The mistake is to turn evolutionary mismatch into a diagnosis of civilization and prehistory into a therapeutic brand.

What older Marxist literature called "primitive communism" was not one social system. Mobile forager societies varied in residence, kinship, hierarchy, violence, exchange, ritual, gender relations, and seasonal aggregation. Many did develop active mechanisms for suppressing would-be dominators: ridicule, mobility, food sharing, coalition formation, and the ability to leave an overbearing camp. Those mechanisms are psychologically interesting because they distribute what modern organizational research calls control. A person surrounded by known others, embedded in reciprocal obligations, and able to walk away from a domineering individual occupies a different stress ecology from a tenant facing an anonymous bureaucracy or an employee whose health insurance is tied to a supervisor. Yet small-scale life also concentrates exposure to bereavement, infection, injury, interpersonal conflict, sorcery accusations, status loss, food uncertainty, and the emotional consequences

of being unable to escape a tiny social world. Depression-like states, suicide, psychosis, grief, violence, and substance problems are not inventions of industrial society. Cross-cultural psychiatry has repeatedly shown that severe mental disturbance exists outside the modern West, while the measurement itself is difficult because symptoms, idioms of distress, and thresholds for impairment differ across cultures.

The limited quantitative evidence cautions against romanticism. A small and old cross-cultural study using the Human Relations Area Files found suicide more prevalent in stable agricultural societies than in hunting-gathering societies, and linked variation to norms around shame, pride, and emotional expression (Smith and Hackathorn, 1982). The study is far too small and historically distant to support a grand conclusion. It is useful mainly as a warning: the psychiatric effect of "traditional life" depends on the social rules inside it. Contemporary work with populations such as the Tsimane similarly undermines the claim that physical activity, face-to-face community, and low digital exposure abolish depression. Small-scale societies may protect against some modern stressors while amplifying dependence on health, kin reputation, and local belonging.

Sedentary kin-based villages changed the psychological equation before states became large. Permanence increased attachment to land, intergenerational continuity, and dense mutual aid. It also reduced exit. A feud, accusation, marriage conflict, or status injury could become inescapable because the people who judged you were the same people who controlled work, inheritance, protection, and marriage. From a mental-health perspective, this is a recurring trade-off throughout history: cohesion can buffer adversity, but cohesion without pluralism can become surveillance. Modern evidence on social capital captures part of the same duality. Cognitive social capital - trust, reciprocity, the expectation that others will help - is generally associated with lower risk of common mental disorders, whereas mere structural participation is not uniformly protective and can sometimes burden people who occupy low-status roles

(De Silva et al., 2005; Ehsan and De Silva, 2015). A village can be a safety net and a panopticon at the same time.

Chiefdoms and redistributive hierarchies added another feature: rank became durable. Surplus could be pooled and redistributed in ways that protected communities against local scarcity, but prestige, tribute, and inherited status created a more stable comparison ladder. This does not allow us to diagnose chiefdoms with a psychiatric "profile." It does suggest a mechanism. Humans are sensitive to relative rank, humiliation, exclusion, and the fairness of distribution. The more a system turns temporary success into hereditary standing, the more it can convert ordinary comparison into chronic subordination. Modern research on social comparison and depression is not direct evidence about Bronze Age chiefdoms, but it makes the inference less arbitrary. A meta-analysis of clinical studies found social-comparison evaluations strongly associated with depression and anxiety, while research on relative deprivation shows that perceived disadvantage can matter beyond absolute deprivation (McCarthy and Morina, 2020).

Early agrarian states added taxation, corvee labor, debt, armies, and legal status. They also created granaries, roads, institutions, markets, writing, and forms of public order that could reduce other risks. The psychiatric question is therefore never whether complexity is "bad." It is which risks complexity removes and which dependencies it creates. A peasant protected from raiding by a state but one failed harvest away from debt bondage may gain safety on one axis and lose it on another. A system that stores grain can reduce famine; a system that confiscates grain can create it. The same administrative capacity can become protection or coercion.

Debt bondage and household slavery are the first systems in this comparison for which the language of domination becomes unavoidable. Ancient forms varied enormously. Some debt servants could eventually exit; some enslaved people could own limited property or be manumitted; others were treated as transferable property. We do not possess diagnostic

surveys of ancient slaves, and it would be false precision to assign them rates of PTSD or depression. What we do know from modern trauma science is that uncontrollable threat, violence, separation from family, sexual coercion, sleep disruption, humiliation, and inability to escape are potent risk factors for psychiatric distress. Modern survivors of trafficking and forced labor, a morally and institutionally different population but a closer analogue than free modern workers, show high burdens of PTSD, complex PTSD, depression, and anxiety in systematic reviews. The inference to ancient bondage is therefore mechanistic rather than epidemiological: systems that legalize ownership of persons institutionalize nearly the full set of exposures known to produce trauma-related disorder.

The broader lesson from pre-state and early-state systems is that no social arrangement eliminates vulnerability. Small societies can offer belonging and meaningful agency while making reputation existential. Agrarian states can create public goods while converting scarcity into debt and hierarchy. The mind is not adapted to one political constitution. It is adapted to regulate threat, attachment, rank, uncertainty, reciprocity, and control. Political systems matter psychiatrically because they distribute those variables.

Hierarchy Becomes an Institution

Greco-Roman chattel slavery made domination legally explicit and economically scalable. Enslaved people could be sold, punished, sexually exploited, separated from children, compelled to labor, and denied civic standing. Elite Roman texts sometimes praise humane treatment, but benevolence within a property relation does not remove the relation's central psychological fact: another person possesses enforceable power over your body and future. The most plausible psychiatric fingerprint is not a single diagnosis but a cluster of chronic threat responses - hypervigilance, despair, dissociation, traumatic stress, substance use where intoxicants are available, and the injuries of learned powerlessness. We cannot verify prevalence retrospectively. We can verify that the causal ingredients are real.

The Spartan helot system offers a different configuration: a subordinated population living under militarized domination, with institutionalized intimidation and periodic violence. Here the system's function depended not merely on extracting labor but on maintaining a political psychology of fear. A society can use terror instrumentally without making every victim clinically ill. Indeed, functioning under threat often requires precisely the opposite: vigilance, emotional control, coalition-building, and strategic obedience. The error is to identify adaptation with health. Hypervigilance may be functional in an environment where arbitrary violence is possible and costly after the threat recedes. Political systems can therefore manufacture symptoms by making the symptoms adaptive.

Plantation racial slavery combined property in persons with heredity, race, family separation, sexual violence, labor discipline, and a legal order designed to close exit across generations. Its downstream psychological effects continued after formal emancipation through violence, segregation, dispossession, and discrimination. Direct psychiatric data on enslaved populations are sparse and conceptually contaminated by the racist medicine of the period. One of the most revealing artifacts is Samuel Cartwright's 1851 invention of "drapetomania," the alleged disease causing enslaved Africans to flee. It demonstrates a separate mechanism from trauma: a social order can medicalize resistance to itself. The institution both creates conditions of suffering and offers a diagnostic language that mislocates the problem in the person. Drapetomania is not an argument that modern diagnosis is fraudulent. It is an argument that psychiatry requires an external standard of dysfunction that cannot simply be obedience to prevailing institutions.

Caste-stratified orders produce less overt coercion than slavery in many everyday interactions while making rank socially pervasive. The psychological burden depends on how strongly caste controls marriage, occupation, residence, dignity, exposure to violence, and access to institutions. A systematic review of caste exclusion in South Asia identifies

stigma, poverty, violence, and inequitable health care as recurring pathways. Again, the evidence is stronger for the exposures than for a unique "caste disorder." A caste system is likely to increase depression, anxiety, trauma, and substance misuse insofar as it makes humiliation chronic and exit improbable. It may also induce defensive forms of identity, solidarity, pride, and collective action that protect mental health. Oppression does not erase agency.

Manorial feudalism and serfdom organized dependency around land, status, and customary obligation. Compared with chattel slavery, serfs often possessed family continuity, customary rights, and some claim to subsistence. Compared with free labor, they faced restricted mobility and inherited obligations. The psychologically relevant variable is not the label "feudalism" but the mixture of predictability and subordination. Custom can reduce uncertainty: rents and duties may be stable, local norms familiar, community dense. The same custom can make one's life course nearly immovable. For some minds, predictability protects; for others, blocked mobility and inherited inferiority may produce hopelessness, resentment, or fatalism. Historical records of melancholy, possession, suicide, alcohol use, violence, and religious distress exist, but diagnostic translation across centuries is hazardous. It is safer to say that feudal systems likely traded market insecurity for status immobility.

Patrimonial and absolutist monarchies intensified the gap between ruler and subject but did not necessarily regulate ordinary life more tightly than local feudal authorities. A strong monarchy could suppress private warfare, standardize law, and improve physical security. It could also make justice dependent on favor, censor dissent, and expose subjects to arbitrary punishment. The psychiatric risk of arbitrary authority is better documented in modern settings: low procedural justice, uncontrollable threat, and political repression are associated with psychological distress. Studies of former East German political prisoners show long-term elevations in PTSD, anxiety, depression, and dissociation decades after imprisonment, and research on non-carceral repression finds elevated

affective and anxiety disorders among people subjected to surveillance, career sabotage, and social marginalization. These data do not prove that monarchy as such causes mental illness. They identify arbitrariness and unappealable power as pathogenic features wherever they occur.

Mercantilist and corporative early capitalism replaced some inherited status with market relations while preserving guild privilege, colonial monopolies, religious discrimination, and coercive labor. Markets can expand exit by allowing people to sell labor to more than one patron. They can also commodify necessities and expose households to price shocks. The psychiatric difference between a feudal obligation and a market dependency is not simply liberty versus oppression. A worker may be legally free yet practically unable to refuse dangerous work. Conversely, cash income can provide a form of exit that customary dependency denies. Money is psychologically important because it can buy distance from a bad landlord, an abusive household, a humiliating employer, or an unsafe neighborhood. The Costello natural experiment in North Carolina is striking precisely because a change in income changed child psychiatric symptoms without any psychotherapy aimed at the children.

Settler-colonial and forced-assimilation regimes add cultural destruction to material domination. Land loss, forced relocation, boarding or residential schools, language suppression, family separation, and institutional discrimination can attack the social structures through which identity and care are reproduced. The evidence here is substantial but must be handled carefully. A systematic review of Indigenous historical trauma studies found many significant associations with adverse health outcomes but also conceptual and methodological inconsistency (Gone et al., 2019). A scoping review of Canadian residential-school research identified sixty-one empirical articles linking attendance to mental distress, depression, substance misuse, stress, and suicidal behavior (Wilk et al., 2017). These findings justify strong claims about specific institutions such as residential schools. They do not justify treating "intergenerational trauma" as a simple biological inheritance or assuming that every

descendant is damaged. Cultural continuity, political recognition, social support, and community resilience are themselves causal forces.

The common denominator across these hierarchical systems is constrained exit. Ownership, caste, serfdom, arbitrary authority, and colonial status differ morally and institutionally, but each can make the person dependent on a relationship they cannot safely leave. That matters because control and escape are central to stress physiology and learned threat. The deepest psychiatric privilege may be the credible ability to say no.

The Age of Ideology: Systems That Claimed the Whole Person

Industrial liberal capitalism did something historically unusual: it dissolved many inherited statuses while making survival increasingly dependent on wages and markets. Nineteenth-century laissez-faire systems expanded legal freedom, urban mobility, trade, and associational life, but also exposed workers to industrial injury, child labor, overcrowding, job loss, and cycles of boom and bust. The mental-health signature is therefore mixed. Urban anonymity can liberate a person from village surveillance; it can also produce loneliness. Wage labor can provide independence from family and landlord; it can also make dismissal a crisis in housing and food. Modern research on unemployment, job insecurity, high demand, low control, effort-reward imbalance, and workplace bullying gives us evidence about the mechanisms even when Victorian diagnostic records do not. The transition from status to contract did not eliminate dependency. It changed who could impose it and how visible it was.

Paternalist industrial systems and company towns tried to stabilize this volatility by tying housing, welfare, recreation, and sometimes schooling to the employer. This may reduce short-term insecurity while concentrating power. A worker who can lose job, home, credit, and community by crossing one institution possesses less real exit than a nominally free labor contract suggests. The psychological problem resembles what appears

throughout this book: bundled dependency. When one organization controls too many life domains, small conflicts become existential. The same principle applies to modern employment when visas, health insurance, housing, or professional identity depend on a single employer.

Fascist corporatism promised to end class conflict by integrating workers, employers, and the state into a national hierarchy. Its mental-health effects cannot be isolated cleanly from censorship, political violence, militarization, war, and persecution. The psychiatric risk to targets of fascist repression is direct: trauma, grief, fear, imprisonment, exile, and loss. For committed followers, the question is different. Fascism may organize ordinary psychological mechanisms - belonging, status, sacred values, leader idealization, outgroup threat - into a system that rewards aggression and punishes ambivalence. This is not equivalent to saying that fascists are mentally ill. Most political extremism is not psychosis. A society can normalize cruelty through identity and incentives while its participants remain clinically ordinary.

Nazi racial totalitarianism is the strongest historical example of why political evil should not be collapsed into psychiatric diagnosis. The regime built genocide through bureaucracy, ideology, war, and participation by many people who did not meet criteria for mental disorder. Calling Nazism "madness" can actually reduce understanding by turning institutionalized moral catastrophe into an individual clinical anomaly. The psychiatric effects on victims, however, are measurable. A meta-analysis of seventy-one samples and 12,746 participants found Holocaust survivors had substantially more posttraumatic stress symptoms than comparison groups while also showing striking resilience in other domains (Barel et al., 2010). That combination matters. Systems can create enduring trauma without creating total psychological ruin. Resilience is not evidence that the system was harmless.

Stalinist command totalitarianism adds a different mix: political terror, purges, denunciation, forced labor, censorship, chronic uncertainty about permissible speech, and an economy in which the state controlled both

employment and political status. The mental-health mechanisms include trauma among direct victims, hypervigilance under unpredictable repression, moral injury among coerced participants, family silence, and the social epistemology of living where public statements may contradict private knowledge. Political psychiatry added a particularly perverse loop. In the Soviet Union, dissidents were at times diagnosed and confined in psychiatric hospitals, turning diagnostic authority into a technology of political legitimacy. Historical scholarship documents systematic abuse of psychiatry for repression. This does not show that psychiatry is inherently repressive; it shows what happens when a clinical institution loses independence from political power.

Maoist mass-mobilization communism intensified the attempt to reorganize not only property but belief, identity, education, and everyday relationships. The Cultural Revolution's "sent-down" movement offers rare quasi-experimental evidence. About seventeen million urban youths were compelled to relocate to rural areas for hard labor and political reeducation. A regression-discontinuity study exploiting cohort eligibility found that rustication increased the likelihood of mental disorders about forty years later (Gong, Lu, and Xie, 2020). The result is valuable because it is not simply a retrospective story about an evil period. It identifies a policy-induced adolescent exposure with measurable long-run mental-health consequences. Other research on the era is more mixed, reminding us that individuals adapted differently and that selection and post-policy opportunities matter.

The Great Chinese Famine provides an even starker demonstration of how political-economic systems can enter psychiatric epidemiology through nutrition. Studies of prenatal famine exposure reported roughly doubled risk of adult schizophrenia in exposed birth cohorts. Famine is not a psychiatric intervention, and its causes are contested across demography, economics, and political history. Yet the causal chain is plausible and partly biological: a political-economic catastrophe changes maternal nutrition and stress, which changes neurodevelopmental risk decades later. This is

the opposite of a social-construction argument. The social event becomes biological vulnerability.

Late bureaucratic state socialism, after the peak terror years, often replaced acute political danger with scarcity, restricted mobility, surveillance, career dependency, and the low-grade stress of institutions that controlled housing, employment, travel, and expression. The East German evidence is especially useful because reunification made long-term follow-up possible. Former political prisoners showed high lifetime and current PTSD as well as anxiety and dissociation, and later studies found large differences in anxiety and depression compared with matched population samples. Research on people exposed to "soft repression" - surveillance, professional sabotage, and marginalization without imprisonment - also reports elevated psychological distress. A system need not torture everyone to produce psychiatric effects. It can make a minority pay a very high psychological price for dissent while teaching the majority to manage speech and trust.

Market socialism and worker self-management complicate the picture. Yugoslav-style self-management, cooperative ownership, and related proposals try to reduce alienation by giving workers voice and a share in governance while retaining markets. Mental-health evidence specific to the regime form is thin. Mechanistically, greater autonomy and procedural voice should reduce some work-related distress; modern occupational research consistently associates low control, high demands, bullying, low justice, and effort-reward imbalance with common mental disorders. Yet participatory systems can create their own burdens: endless meetings, factional conflict, responsibility without resources, and local majorities capable of humiliating minorities. Voice is protective only when it is consequential and bounded.

Military-authoritarian developmental regimes trade political pluralism for order, investment, and state-led growth. Where they reduce poverty, violence, or unemployment they may improve some determinants of mental health; where they suppress dissent, permit arbitrary detention, or

make status dependent on political conformity they create others. The psychiatric question cannot be answered by regime labels alone. A competent authoritarian state that provides housing and safety may outperform a chaotic democracy on some population stressors while imposing extreme costs on dissidents. This possibility is precisely why mental health cannot serve as the sole measure of political legitimacy. A tranquil population is not necessarily a free one.

Across fascist, communist, and military-authoritarian systems, the greatest mistake would be to search for one "totalitarian personality disorder." These systems differ profoundly. What they share at their most coercive is the capacity to make social reality monopolistic. Employment, information, public status, and legal protection can converge under institutions that punish exit and dissent. The psychiatric risk arises from fear, helplessness, forced participation, epistemic isolation, and trauma. The political risk arises from something different: ordinary people can become agents of persecution without becoming mentally ill.

Democratic Capitalisms: Freedom, Security, and the Price of Exit

Modern democratic capitalism is not one system but a family of arrangements that distribute market risk, social insurance, bargaining power, housing, education, and health care differently. This matters because "capitalism causes depression" and "capitalism protects freedom" are both too coarse to be scientifically useful. Markets are compatible with a Nordic welfare state, an American employer-insurance system, a German social-market model, and developmental states in East Asia. If mental health differs across them, the causal variables are likely to be the institutions layered around markets rather than the existence of exchange itself.

Liberal welfare capitalism in the Anglo tradition combines market allocation with targeted or moderately universal social protection. Its advantages include pluralism, mobility, political rights, and multiple

sources of identity. Its psychiatric vulnerabilities include high exposure to job loss, expensive housing, debt, and the risk that medical care, education, or retirement security become strongly tied to income. Comparative welfare-regime studies find cross-national differences in depression and mental well-being, but the patterns are not simple enough to crown a winning regime. Chung and colleagues' analysis of twenty-six countries showed differences across welfare families and important interactions with gender, while other European studies found welfare-regime categories did not explain all gender gaps in well-being. Macrosocial epidemiology is vulnerable to confounding, measurement differences, and small numbers of countries. It is evidence, not a randomized trial of constitutions.

Neoliberal democratic capitalism is a more specific hypothesis about the balance of institutions: weaker unions, privatization, financialization, flexible labor markets, targeted welfare, and stronger reliance on individual market participation. The plausible mental-health risks are insecurity, indebtedness, status competition, weak bargaining power, and the individualization of failure. The plausible benefits are choice, innovation, mobility, and exit from rigid institutions. The evidence is strongest at the level of these mechanisms. Job insecurity and unemployment predict depressive symptoms prospectively; low job control and workplace injustice are associated with common mental disorders; debt is associated with worse mental health; poverty and depression have bidirectional causal links. The evidence is much weaker for the global sentence "neoliberalism causes mental illness," because countries classified as neoliberal differ in family structure, health care, housing, immigration, inequality, and measurement. The book should be provocative where the evidence permits and boring where the evidence requires.

Social-market and ordoliberal capitalism attempts to preserve markets while constructing rules that prevent monopoly, provide social insurance, and embed labor relations in institutions of bargaining and codetermination. Its likely mental-health advantage is not ideological moderation as such but de-risking: health care and unemployment

insurance reduce the number of life domains that collapse after job loss. Its possible cost is administrative complexity and insider-outsider divisions, where protected core workers coexist with precarious outsiders. A welfare state can reduce market anxiety and simultaneously produce bureaucratic anxiety if benefits are opaque, conditional, humiliating, or difficult to appeal.

Nordic social democracy goes further by combining markets with universalistic social protection, high taxation, active labor-market policies, extensive public services, and relatively compressed income distributions. It is tempting to declare this the psychiatric winner. The data justify a more modest statement. Welfare generosity can buffer the mental-health consequences of unemployment, and broader social protection can reduce material insecurity. Cash-transfer trials in low- and middle-income countries show small average reductions in depression and anxiety, with stronger effects in unconditional than conditional programs and attenuation after support ends (Wollburg et al., 2023). Comparative studies also often associate more comprehensive welfare states with better health and narrower inequalities. Yet Nordic countries still have depression, anxiety, suicide, loneliness, substance misuse, and severe mental illness. Universal security removes some causes; it does not remove the human condition. High expectations, social comparison, winter light, alcohol cultures, family change, and other factors continue to matter. A good system can lower the base rate of preventable distress without producing psychological utopia.

Income inequality deserves special attention because it connects economic design to status psychology. A meta-analysis found a pooled risk ratio of about 1.19 for depression in higher-inequality compared with lower-inequality settings, but heterogeneity was extremely high and causal mechanisms remain disputed (Patel et al., 2018). Inequality can operate materially through neighborhood resources and political power, relationally through trust and segregation, and psychologically through comparison and social defeat. It can also be a proxy for many other

institutions. The finding is strong enough to take seriously and too weak to treat as a universal law.

Envy belongs here, but not in the way political polemic often uses it. Envy is a normal emotion produced by upward comparison when another person's advantage is self-relevant. Psychologists distinguish benign envy, which can motivate improvement, from malicious envy, which motivates derogation or a desire to pull the other person down. Social comparison is strongly associated with depression and anxiety in clinical samples. On social media, systematic reviews find links among upward comparison, envy, and depressive symptoms, but directionality is not clean; in some longitudinal work depression predicts later comparison and envy rather than the reverse. This matters politically. A system with conspicuous inequality may provide more triggers for painful comparison, but it is scientifically reckless to treat envy as the master disease of capitalism or egalitarian politics. Envy can become socially destructive without being psychiatric illness.

East Asian developmental capitalism represents another combination: markets, high investment, education competition, strong states, family obligations, long working hours in some sectors, and varying welfare provision. Rapid growth can reduce poverty and improve health while competitive schooling and work cultures can generate stress, sleep loss, and status pressure. The policy lesson is that rising GDP does not automatically settle psychiatric welfare. Development changes the composition of risk. Infectious disease, hunger, and physical insecurity may decline while loneliness, educational pressure, overwork, and aging-related isolation rise.

The most useful way to compare democratic capitalisms is therefore not by asking which ideology makes people happy. Ask how easily a person can refuse a bad situation without losing everything else. Can an employee leave without losing health care? Can a tenant move without losing access to school? Can a parent take time off without risking eviction? Can a patient contest a bureaucratic decision? Can an entrepreneur fail without

permanent debt? Can a dissenter remain employable? The psychiatric variable hidden inside many political debates is exit capacity.

Democracy adds something markets cannot supply: voice. Elections, unions, courts, media, civil society, and administrative appeal procedures are imperfect technologies for making power contestable. There is ecological evidence linking rights, government quality, and population well-being, but mental-health-specific causal evidence is limited. The mechanism is nevertheless credible. Procedural justice matters even when outcomes are unfavorable. People tolerate constraint better when they are heard, given reasons, treated consistently, and able to appeal. Democratic institutions may therefore protect mental health not by producing permanent satisfaction but by reducing the experience of unanswerable power.

This gives democratic capitalism a distinctive failure mode too. Formal rights can coexist with practical powerlessness. A person may have legal freedom to quit, move, sue, vote, and complain while lacking money, time, information, health, or social support to use those rights. The psychological difference between nominal exit and credible exit is enormous. Systems should be judged not only by the rights printed in law but by how many people can actually exercise them without catastrophic loss.

Futures Not Yet Tried: From Minimal States to Engineered Abundance

The final systems in this comparison are intellectually interesting precisely because the evidence is weakest. They are proposals, fragments, or partial experiments rather than stable historical regimes. The correct scientific posture is not to dismiss them for lacking data - every institution was once untested - but to distinguish prediction from observation. Here the book moves from psychiatric epidemiology into constrained speculation. The constraints are the mechanisms established elsewhere: material security,

autonomy, predictability, belonging, status, discrimination, violence, social capital, voice, exit, and the concentration of unappealable power.

A libertarian minimal state seeks to confine government largely to protection against force, theft, fraud, and breach of contract, leaving welfare, health care, education, and much social provision to private choice and voluntary institutions. No modern country is a clean test. The psychiatric promise is substantial autonomy: fewer coercive bureaucracies, more pluralism, and potentially easier exit from state institutions. The psychiatric risk is equally obvious: if health care, disability support, housing, and unemployment protection depend mainly on private resources, people with low income, chronic illness, or weak family networks may face greater material insecurity. Concentrated private power can also replace public power; a monopolistic employer, landlord, insurer, or platform can be psychologically coercive without being a state. The mental-health outcome would likely depend less on the smallness of government than on whether civil society and markets genuinely provide redundant routes of support and exit.

Anarcho-capitalism extends this logic toward private provision of law, security, arbitration, and infrastructure. Its defenders imagine competition among institutions; its critics imagine privatized domination. There is essentially no direct population mental-health evidence because the system has not existed at scale in the proposed form. The key psychiatric question is whether exit would be real for people with little bargaining power. If every protection is contractual, then cognitive capacity, wealth, family resources, and information become determinants of safety. On the other hand, polycentric provision could reduce the single-point failure of state bureaucracy. Both conclusions are hypotheses.

Localist and polycentric systems move in a different direction. Municipalism, confederalism, cooperative ownership, local currencies, participatory budgeting, and some forms of libertarian socialism aim to shorten the distance between decisions and the people affected by them. The mental-health case is plausible: participation can increase agency, local

trust, and social capital; cooperative institutions can provide belonging and reduce alienation. The evidence for social capital is suggestive rather than decisive. Reviews find cognitive social capital associated with lower common mental disorder, while intervention studies remain limited and heterogeneous. Localism also has a dark side. Small communities can be exclusionary, nepotistic, conformist, and difficult to escape. A national bureaucracy may be cold, but a local majority can know exactly whom to punish. The psychologically best version of localism therefore requires nested levels of appeal and rights that a locality cannot revoke.

Dark Enlightenment or neoreactionary neocameralism is almost the inverse experiment. In its well-known online formulations, democracy is replaced by authoritarian, often corporate-style sovereignty; residents are treated more like customers than citizens; "voice" is minimized and "exit" is supposed to discipline rulers through competition among jurisdictions. There is no clean regime-level mental-health evidence because the model has not been implemented as a coherent system. Its psychiatric prediction can nevertheless be derived from Hirschman's distinction between exit and voice. For affluent, mobile, informed residents, multiple competing city-states might make exit unusually credible. For children, disabled people, the poor, caregivers, prisoners, people with deep local attachments, or anyone whose assets and social ties cannot move quickly, exit can be fictitious. If voice is deliberately weak, the system risks converting ordinary administrative conflict into unappealable dependency. Research on political repression and procedural justice makes concentrated, uncontestable authority a serious mental-health concern even before one debates democracy on moral grounds.

Neoreaction also illustrates why efficiency is not synonymous with psychological safety. A well-run hierarchy can reduce crime, delays, and uncertainty. It can also make belonging conditional on the ruler's objectives. The absence of political conflict may reflect competence or suppression. Mental-health metrics alone cannot distinguish them. A population can report low distress because material life is secure, because

dissenters left, because dissent is dangerous to report, or because a narrow culture defines distress differently. Any psychiatric evaluation of authoritarian proposals must therefore include rights, measurement validity, and the fate of people who cannot exit.

The Venus Project and related resource-based economy proposals imagine a much more egalitarian technological future. The official project proposes eliminating money and debt, treating resources as a common heritage, using automation and scientific planning to provide material abundance, and eventually testing design principles in an experimental research city. Nothing close to this has been evaluated at population scale, so claims that it would eliminate crime, stress, or mental illness are advocacy, not evidence. Yet its mental-health hypothesis is worth taking seriously. Removing debt, unemployment anxiety, housing insecurity, and scarcity competition could reduce several established risk pathways. If basic goods truly became unconditional, the system might create an unusually high floor of psychological security.

Its risk is the mirror image of market insecurity: technocratic concentration. A resource system requires decisions about needs, scarcity, allocation, measurement, infrastructure, and acceptable trade-offs. If these decisions are made by transparent, contestable, plural institutions, technology could reduce arbitrary power. If they are delegated to a centralized expert or computational system with no meaningful appeal, the society could become a high-tech total institution. The Venus Project's own emphasis on computerized resource models and expert coordination therefore raises a question rather than an objection: who can challenge the model when the model is wrong about a person? A psychiatric system has learned painfully that benevolent expertise without appeal can become coercive.

Technocracy more generally shares this problem. Evidence-based administration can reduce corruption and magical thinking. It can also confuse measurable variables with human values. Mental health is particularly vulnerable because optimization targets can become

normative. If an algorithm learns that protest, risk-taking, sexuality, unconventional beliefs, or intense religious practice correlate with distress, a technocratic system might be tempted to reduce the behavior rather than understand its meaning. A sane society cannot define health as compliance with its own objective function.

Universal basic income and universal basic services provide a more testable bridge between existing welfare states and post-scarcity proposals. Cash-transfer experiments do not show utopia, but they do show small average reductions in depression and anxiety. The more interesting result is what does not happen: recipients do not require psychological reeducation before money can help. A structural change can alter mental health directly through choices, conflict, nutrition, time, and exit. Yet the effects are modest and can fade when transfers end. The lesson is not that cash cures depression. It is that economic architecture belongs inside the causal model.

The strongest future system may therefore be less a named ideology than a design principle: multiply routes of exit without destroying belonging; increase voice without making every decision exhausting; guarantee a material floor without making support humiliating; decentralize where local knowledge matters while preserving higher-level rights; use technology to reduce scarcity without creating an epistemic monopoly; allow inequality where it motivates creation but prevent rank from determining dignity and survival; protect people from violence without treating dissent as pathology.

No historical society has solved all of these tensions. That may be the central finding of this chapter. Political systems are not psychiatric treatments. They are distributions of vulnerability and protection. Every system removes some stressors, creates others, and decides who bears the residual risk.

Institutions, Status, and Chronicity

The Hospital That Made Symptoms

The asylum was born from a humane idea and repeatedly became its opposite. Reformers in the late eighteenth and nineteenth centuries argued that people called mad should not be chained in jails, displayed, abandoned, or treated as animals. “Moral treatment” promised order, dignity, occupation, calm, and relationship. In small settings with committed staff, some patients improved. As institutions expanded, overcrowding, poverty, custody, and professional power transformed refuge into warehousing.

This history resists a simple villain. Families sometimes sought admission because no community support existed and danger was real. Hospitals protected people from starvation, violence, and their own suicidal impulses. Staff worked inside impossible conditions. Medication later reduced acute psychosis for many. At the same time, institutions removed liberty, identity, possessions, privacy, ordinary roles, and the expectation of a future. An organization created to treat illness could become one of the environments maintaining disability.

Erving Goffman described the mental hospital as a “total institution,” a place in which sleeping, working, and daily life occur under one authority, in the company of similarly situated people, according to an imposed schedule. His analysis was sociological rather than a clinical trial, and some hospitals did not fit the model. Its enduring value lies in showing how institutional behavior can be misread as personal pathology. Passivity may

be adaptation to a setting where initiative has no consequence. Hoarding may respond to the removal of possessions. Withdrawal may protect against surveillance. The institution observes the behavior it helped evoke and records it as evidence of the patient's condition.

John Wing and George Brown (1970) supplied empirical support through their comparative study of British mental hospitals. They found that the poverty of the social environment was associated with what they called "clinical poverty": blunted affect, reduced speech, social withdrawal, and limited activity among long-stay patients with schizophrenia. As hospital environments became more stimulating and less restrictive between 1960 and 1968, some symptoms improved. The data did not show that hospitals caused schizophrenia. They suggested that institutional conditions could amplify the negative-symptom-like presentation through which chronic schizophrenia was recognized.

This distinction is crucial. The institution may not create the original psychosis, yet it can create part of the chronic phenotype. A patient enters with hallucinations and disorganization, then loses ordinary decisions, work, intimacy, and social expectations. Years later, the absence of initiative is attributed entirely to disease. Biological illness and institutional adaptation have fused.

Deinstitutionalization was intended to reverse this process. New medications, civil-rights litigation, changing professional beliefs, and the cost of large hospitals drove the closure of beds across many countries. In principle, people would live in communities with outpatient care, housing, income, and rehabilitation. In practice, the hospital often disappeared before the replacement system existed. Families carried unsupported responsibility. Some people cycled through emergency departments, shelters, and short admissions. Others entered nursing homes or prisons. The result is sometimes called transinstitutionalization.

It is too simple to say that closing asylums caused homelessness and incarceration. Housing markets, welfare policy, drug enforcement, poverty, service fragmentation, and the rise of mass incarceration were independent

forces. Many former patients lived successfully in the community. Large hospitals had caused serious harm. The policy failure was not freedom; it was freedom without infrastructure.

Prisons reveal the extreme version. People with mental illness are overrepresented in criminal-justice systems for reasons that include poverty, substance use, inadequate care, policing practices, and the criminalization of survival behavior. Once incarcerated, symptoms may lead to rule violations and placement in segregation. The response to dysregulation becomes greater deprivation.

Research on solitary confinement is methodologically difficult because people placed in isolation differ from other prisoners. They may have more severe illness or behavioral problems before placement. Even with this selection problem, systematic reviews and higher-quality studies associate solitary confinement with anxiety, depression, paranoia, self-harm, and mortality. Luigi and colleagues' review (2020) found that stronger evidence generally pointed toward adverse psychological effects. Reiter and colleagues documented distress among prisoners held in isolation. A more recent meta-analysis also reported increased mood symptoms. The exact magnitude is uncertain; the direction is difficult to defend as benign.

Isolation is a social intervention with an almost perfectly inverted therapeutic design. It removes relationships, meaningful activity, sensory variation, autonomy, and temporal structure while increasing uncertainty and surveillance. For a person already vulnerable to psychosis or trauma, the environment can become symptom-generating. The prison may then interpret deterioration as proof that more control is required.

Psychiatric hospitals can reproduce parts of this logic through involuntary admission, seclusion, restraint, forced medication, and opaque decision-making. These practices are sometimes used during genuine emergencies to prevent imminent harm. Abolitionist arguments can underestimate the terror of untreated mania, catatonia, suicidal depression, or psychosis and the burden placed on families when care refuses every form of coercion. Yet necessity is not evidence of harmlessness.

Systematic reviews of involuntary inpatient treatment find mixed and limited evidence on benefits, alongside increased subjective coercion, reduced satisfaction, avoidance of later care, and possible trauma. Many studies are observational and confounded by severity; people admitted involuntarily are often more acutely ill. It is therefore difficult to compare their outcomes fairly with voluntary patients. The honest conclusion is uncomfortable: coercion may sometimes save a life, can also injure the person and therapeutic relationship, and has a weaker evidential base than its institutional normality suggests.

The experience of coercion depends not only on legal status but on procedure. People may feel coerced during a formally voluntary admission and respected during an involuntary one. Explanation, voice, proportionality, cultural understanding, continuity, and the possibility of appeal matter. A locked door is not the only source of power. A clinician controls diagnosis, discharge, benefits documentation, access to housing, and credibility. Even benevolent decisions occur within asymmetry.

This asymmetry can produce epistemic injustice. A person with a psychiatric diagnosis may have reports of pain, abuse, medication effects, or discrimination discounted as symptoms. Suspicion can be pathological, but institutions can also be untrustworthy. The patient faces a trap: anger at not being believed becomes evidence of instability; calm persistence becomes evidence of rehearsed delusion. A diagnostic system without mechanisms for credible disagreement can become self-sealing.

Medication adds another difficult layer. Antipsychotics, lithium, antidepressants, stimulants, and other treatments can be life-changing. They also cause adverse effects and do not work for everyone. When side effects such as sedation, emotional blunting, weight gain, akathisia, sexual dysfunction, or cognitive slowing are ignored, the treatment system may create disability attributed to illness. Conversely, abrupt discontinuation can cause withdrawal or relapse that critics wrongly treat as proof the original illness was invented. The causal story requires attention to both treatment benefit and treatment burden.

Fragmentation maintains illness in less dramatic ways. One service treats symptoms, another controls housing, another assesses disability, another manages substance use, and none controls the whole trajectory. Eligibility rules are diagnosis-specific while lives are not. A person may be “too ill” for employment support, “not ill enough” for intensive care, “noncompliant” because transport failed, and “treatment resistant” because treatment never addressed safety. Institutional failure is converted into a patient adjective.

Online critiques of psychiatry are often strongest at this level. People describe not a belief that suffering is unreal, but the experience of being processed by systems that have incentives to stabilize risk rather than restore a life. They report being asked whether they take medication more often than whether they have somewhere safe to sleep. They describe clinical improvement measured as fewer disruptions to institutions. These accounts are selective and cannot establish rates of abuse. They identify outcomes that formal studies often fail to measure.

A hospital can also be the opposite of an illness machine. It can provide safety, sleep, nourishment, diagnosis of a medical condition, relief from psychosis, and a relationship that survives crisis. Community teams can reduce hospitalization. Peer support can restore agency. Advance directives and supported decision-making can reduce conflict. The relevant distinction is not institution versus no institution, but whether the system treats the person as a risk object to be managed or as an agent whose life must become possible again.

The historical asylum teaches a design principle. Environments create behavior, and systems with near-total interpretive power can mistake their own effects for patient traits. Any institution that controls a person’s world must study not only what the person brings into it, but what the institution brings out of the person.

The Diagnosis That Changes the Patient

A diagnosis is a sentence in both meanings of the word. It is a statement about what may be happening, and it can determine what happens next.

For many people, receiving a diagnosis ends a period of frightening uncertainty. It provides language, treatment, insurance coverage, legal protection, community, and the knowledge that others have survived something similar. For others, it feels like an administrative transformation from person into prognosis. The same label can be a key and a cage.

Classic labeling theory argued that psychiatric labels can produce a “career” of mental illness by changing how others respond and how the person understands the self. Strong versions implied that social reaction creates the disorder. Critics correctly observed that severe symptoms often precede labeling and have consequences independent of stigma. Modified labeling theory made a more defensible claim. People learn cultural beliefs about mental illness before diagnosis. Once labeled, they anticipate rejection, conceal, withdraw, or devalue themselves; these responses then worsen social and clinical outcomes.

Research on self-stigma supports parts of this pathway. Internalized stereotypes are associated with lower self-esteem, reduced hope, poorer quality of life, and less pursuit of work or treatment. Causality is difficult because more severe illness can increase both stigma and poor outcomes. Longitudinal studies and intervention research nevertheless suggest that stigma is not merely an opinion attached to an unchanged disease. It can become a maintenance mechanism.

The social meaning of a diagnosis depends on the causal story attached to it. Public campaigns often use biological explanations to reduce blame: mental illness is an illness like any other. Meta-analytic research by Kvaale and colleagues (2013) and later reviews found a paradox. Biogenetic explanations can reduce personal blame and sometimes increase willingness to seek help, but they can also increase perceived dangerousness, social

distance, and pessimism about recovery. The person is forgiven because the condition is imagined as essential and permanent.

Psychosocial explanations can humanize by connecting symptoms to experience, but they carry their own risks. They may imply that the person or family caused the illness, that recovery should follow insight, or that medication is illegitimate. The most stigma-reducing account may be neither “broken brain” nor “bad environment,” but a nonessentialist model of interacting vulnerabilities and changeable pathways. Scientific complexity can be morally useful.

History demonstrates what happens when diagnostic authority fuses with social norm. In 1851, the American physician Samuel Cartwright proposed “drapetomania” to explain why enslaved people fled captivity. The pseudo-diagnosis converted a rational escape from violence into pathology and prescribed domination as treatment. It was not a mistaken hypothesis within an otherwise neutral science. It was slaveholding ideology wearing medical language.

The Soviet use of “sluggish schizophrenia” against political dissidents provides a later case. The concept referred to a mild or slowly progressive form of schizophrenia that could be diagnosed without the florid symptoms expected elsewhere. Its elasticity allowed dissent, reformist conviction, or conflict with authority to be interpreted as signs of disease. Not every Soviet use of the category was political, and debates about mild schizophrenia existed internationally. The state’s coercive use of diagnosis, however, demonstrated how a category with vague boundaries can become a technology of repression.

Homosexuality’s removal from the DSM is more complex and more instructive. The APA classified homosexuality as disorder until 1973, when research, internal professional change, and gay activism forced reconsideration. Evelyn Hooker’s work had shown that clinicians could not reliably distinguish psychologically healthy gay men from heterosexual controls on projective tests. Activists exposed how the diagnosis legitimized discrimination and how gay psychiatrists had to hide within the profession.

The final decision involved debate and voting, which critics used to claim that science had been replaced by politics. The better interpretation is that political action exposed the values and weak evidence already embedded in the category.

The lesson is not that diagnostic truth is decided by protest. It is that affected people may possess evidence about harm and functioning that institutions systematically exclude. A classification system becomes more scientific, not less, when it can revise itself after such evidence enters.

The history of post-traumatic stress offers the reverse case. Political organization by veterans and survivors helped create a diagnosis that recognized external violence rather than pathologizing weakness. Social influence corrected underdiagnosis rather than producing false disease. Diagnosis is not inherently oppressive or liberating. Its effect depends on what it names, what evidence supports it, and what institutions do with the name.

Modern diagnostic manuals aim to improve reliability by specifying symptom criteria. Reliability is valuable: clinicians should not assign entirely different categories to the same presentation. But reliable agreement can still concern an invalid construct, and categories built from symptom clusters remain heterogeneous. Two people can meet criteria for major depression with few symptoms in common. A diagnosis often summarizes a syndrome rather than identifying a single mechanism.

This is why reification is dangerous. Once a category has a name, it begins to act like an entity that explains the symptoms used to define it. A person sleeps poorly, loses pleasure, feels worthless, and slows down because they “have depression”; the evidence for depression is that they sleep poorly, lose pleasure, feel worthless, and slow down. The circularity is not fatal if the diagnosis predicts course or treatment response. It becomes misleading when the label substitutes for causal inquiry.

Diagnosis also interacts with institutions beyond medicine. Schools, insurers, courts, employers, disability systems, and families use labels for

different purposes. The same ADHD diagnosis can unlock educational support, justify stimulant treatment, alter parental expectations, attract stigma, or conceal a classroom design that fails many children. A schizophrenia diagnosis can guide effective treatment and simultaneously make a landlord or employer retreat. Social consequences then become part of the disorder's functional burden.

Family research shows how relational response alters course. High “expressed emotion,” especially criticism, hostility, and emotionally overinvolved behavior, predicts relapse in schizophrenia and other conditions. Butzlaff and Hooley’s meta-analysis (1998) found a robust association, and family psychoeducation has repeatedly reduced relapse and hospitalization. These findings do not mean families cause schizophrenia. They show that the emotional climate around a vulnerable person changes recurrence risk.

The concept has sometimes been misused to blame relatives, especially mothers, reviving earlier theories of the “schizophrenogenic mother.” Families often respond to terrifying symptoms with little support and may themselves be exhausted or traumatized. Effective family interventions do not prosecute them. They provide information, communication skills, crisis planning, and support. The system changes the family environment by helping the family survive the illness.

Receiving a diagnosis can also transform autobiography. A person rereads childhood, relationships, and failures through a new category. This can generate coherence and self-compassion. It can also create diagnostic overshadowing, in which every emotion becomes symptom and every conflict becomes pathology. Public discussions about receiving a schizophrenia diagnosis contain both relief and injury: recognition, access, and explanation alongside disagreement, stigma, and fear of a fixed future. Qualitative research on self-diagnosis finds similar ambivalence (Underhill & Foulkes, 2025). Online labels create communities but can encourage premature closure around one explanation.

Antipsychiatry forums are often caricatured as denying that mental illness exists. Some participants do. Many make a narrower claim: suffering is real, but psychiatric categories do not map neatly onto biological diseases; treatment can harm; and social conditions are ignored. Other users in the same spaces defend the reality of biological vulnerability, describe benefit from medication, or reject the idea that every psychosis is a political metaphor. The disagreement inside these communities is evidence against treating “lived experience” as one voice.

The strongest patient criticism concerns epistemic monopoly. A clinician knows comparative diagnosis, risk, and treatment evidence. A patient knows the phenomenology of the experience, the meaning of side effects, and the conditions of life. Neither knowledge is complete. Systems become dangerous when one form automatically invalidates the other.

A good diagnosis should behave like a provisional model. It should compress information, open access to useful interventions, and remain revisable when evidence changes. It should not claim more causal knowledge than it contains. It should distinguish a person from a syndrome and a syndrome from its history. It should make new questions possible rather than end them.

Social systems can create illness through diagnosis in two different senses. They can misclassify disapproved difference as disease, as history repeatedly shows. More subtly, they can attach consequences to a valid diagnosis that worsen isolation, hope, work, housing, and care. The label does not invent the original suffering, but it changes the environment in which suffering either resolves or becomes a social identity.

A diagnosis is therefore not merely true or false. It can be approximately valid and badly used, imperfect and lifesaving, scientifically grounded and socially damaging. The task is not to abolish names. It is to prevent names from becoming destinies.

Exclusion, Status, and Social Defeat

Schizophrenia appears in every society that has looked carefully for it, but not at the same rate in every place or group. This variation is among the strongest reasons to reject a purely invariant account of severe mental illness. If biology were the whole story and relevant biological risks were evenly distributed, incidence should be more stable than it is. Instead, psychotic disorders cluster by urbanicity, migration, minority position, neighborhood deprivation, and social fragmentation.

A systematic review and meta-analysis of incidence studies published in 2018 estimated higher rates of psychosis among men, urban residents, immigrants, and people living in lower socioeconomic areas. The pooled incidence-rate ratio for immigrants compared with native-born populations was about three, though rates varied greatly across groups and settings (Castillejos et al., 2018). Earlier meta-analysis found a dose-response relationship between urban upbringing and schizophrenia risk (Vassos et al., 2012). These are not small curiosities. They demand an explanation capable of connecting social position to one of psychiatry's most heritable disorders.

Migration itself cannot be treated as a single exposure. People migrate because of war, persecution, work, family, ambition, or colonial history. They differ in age, language, legal status, skin color, wealth, and reception. Elevated psychosis risk is often greater among visible minorities and can persist into the second generation, which weakens explanations based only on selective migration or conditions in the country of origin. The destination matters.

One influential hypothesis is social defeat: the chronic experience of exclusion from the majority group and subordinate status outside one's control (Selten and Cantor-Graae, 2005). The term does not imply that a person consciously feels defeated at every moment. It describes repeated evidence that one is devalued, unsafe, or unable to secure belonging. Such experience may sensitize dopamine systems involved in salience and threat,

although the neurobiological pathway remains a hypothesis rather than a completed causal chain.

The ethnic-density effect provides suggestive evidence. Members of some minority groups have lower psychosis risk when they live among more people of their own group and higher risk when they are more isolated within the local population. A systematic review and multilevel meta-analysis found that lower own-group density was associated with increased psychosis risk, with variation across minority groups (Baker et al., 2021). Genes do not change when a person crosses a neighborhood boundary. What changes may include discrimination, social support, visibility, cultural recognition, exposure to threat, and the probability that unusual experiences are interpreted within a trusted community.

Ethnic density is not a prescription for segregation. A neighborhood with more co-ethnic residents can protect through solidarity while segregation also reflects exclusion and concentrated disadvantage. The same spatial pattern can contain both refuge and injustice. The relevant finding is that minority status is relational. Being one of few can alter daily exposure even when individual income and formal rights are similar.

Racism operates through direct events and institutional patterns. A 2015 meta-analysis of 293 studies found that reported racism was associated most strongly with poorer mental health, including depression, anxiety, and psychological distress. Longitudinal effects were smaller than cross-sectional ones, which is a reminder that common-method bias and current mood can inflate associations (Paradies et al., 2015). Experimental studies offer a narrower causal test. A 2024 meta-analysis of experiments in which participants were exposed to discriminatory treatment or scenarios found immediate adverse effects on mental health and stress-related outcomes (Emmer et al., 2024). Laboratory discrimination cannot reproduce a lifetime of structural racism, but it establishes that discriminatory encounters can cause acute psychological harm.

Institutional discrimination is harder to randomize but sometimes changes abruptly through law. A prospective study used the passage of

state constitutional amendments banning same-sex marriage in the United States as a natural experiment. Lesbian, gay, and bisexual adults living in states that enacted bans showed increased psychiatric morbidity compared with those in states without such changes, while heterosexual adults did not show the same pattern (Hatzenbuehler et al., 2010). The study has limitations, including sample size and the difficulty of isolating legal change from the political campaigns surrounding it. Its strength lies in temporal and group specificity: a public act of exclusion was followed by a targeted change in mental health.

This is how a law can enter a nervous system. It does not need to mention cortisol or depression. It announces who is fully recognized, encourages or restrains hostility, changes expectations about the future, and alters the credibility of protection. Institutional discrimination is not merely a bad opinion held by individuals. It modifies the background probability of humiliation, concealment, and threat.

Status reaches beyond legally defined groups. Humans monitor rank with extraordinary sensitivity. Income matters partly because it buys resources, but relative position also shapes perceived control, respect, and security. The Whitehall studies found a social gradient in health among British civil servants who were not poor in an absolute sense. Lower occupational rank was associated with worse outcomes despite universal access to health care. The mechanisms included work control, behavior, and material conditions rather than one pure status effect, but the gradient challenged the idea that only destitution harms.

Inequality may intensify status comparison and weaken trust, yet claims that income inequality directly causes mental illness remain contested. Cross-national correlations are vulnerable to country differences, measurement choices, and publication bias. Some studies find relationships with depression or psychosis; others find small or inconsistent effects after controlling for individual income. It is safer to say that unequal systems can amplify known mechanisms—segregation,

insecurity, reduced public goods, political power imbalance, and status anxiety—than to treat a national Gini coefficient as a psychiatric toxin.

Humiliation is an undermeasured mechanism because it is hard to standardize and politically embarrassing. People can survive material hardship while retaining dignity and collapse under repeated treatment as disposable. Debt collection, welfare assessments, immigration interviews, policing, workplace ranking, and medical encounters all contain moments in which a person must submit an account of themselves to an authority that can disbelieve them. The effect depends on procedural justice, not only the final decision.

Loneliness provides the negative image of status: not active subordination, but insufficient social recognition. Loneliness is not the same as living alone. It is the perceived gap between desired and available connection. Longitudinal studies show that loneliness predicts later depression and other mental-health problems, while depression also predicts later loneliness. A large British cohort analysis found that loneliness was associated with the onset of new mental-health problems, even after adjustment for baseline factors, although bidirectionality remained important (Mann et al., 2022).

The causal loop is easy to imagine. Rejection increases vigilance. Vigilance makes ambiguous encounters feel dangerous. Withdrawal reduces opportunities for corrective experience. Social skills atrophy, sleep deteriorates, and the remaining interactions carry more pressure. A person becomes lonely partly because of symptoms and symptomatic partly because of loneliness. The system sustains illness by making re-entry costly.

Modern societies contain enormous numbers of contacts while weakening durable membership. A person can communicate continuously and remain unclaimed by any group that notices absence. Employment may be temporary, neighborhoods transient, families dispersed, and communities organized around consumption rather than obligation. This does not prove that traditional societies were mentally healthier. Traditional communities could be coercive, violent, and intolerant, and

historical prevalence estimates are unreliable. It does suggest that connection should be measured by reciprocity and continuity, not message count.

Émile Durkheim's 1897 study of suicide made social integration and regulation central to a phenomenon often imagined as purely individual. His categories and data belong to another era, and later scholarship has revised much of the analysis. The enduring insight is that suicide rates vary systematically with social organization. Individual despair is not randomly distributed across the calendar of institutions. Economic crisis, war, religious community, family structure, and political change can alter population rates even though each death has a singular biography.

Suicide illustrates why average social causation must never erase individual meaning. A rise in a national rate does not tell us why a particular person died. A person's reasons may include pain, psychosis, shame, bereavement, impulsivity, addiction, or a crisis that no macro-variable captures. Population explanation and personal understanding operate at different levels. Policy fails when it substitutes one for the other.

Exclusion can also follow diagnosis rather than precede it. People with severe mental illness are less likely to be employed, more likely to experience poverty and victimization, and often lose social networks. Employers may overestimate risk or underestimate competence. Families may narrow expectations in the name of protection. Services may organize life around illness. Social selection then becomes a maintenance mechanism: the disorder moves the person downward, and the lower position worsens the disorder.

Supported employment offers evidence that this loop can be interrupted. Individual Placement and Support seeks rapid placement in competitive work according to the person's preferences, combined with ongoing support, rather than prolonged pre-vocational training. Meta-analyses across countries find that it improves competitive employment outcomes for people with severe mental illness (Modini et al.,

2016). Employment does not benefit everyone equally, and bad work can harm. The important point is that exclusion is not an inevitable symptom of schizophrenia. It is partly produced by how labor markets and services respond to schizophrenia.

Online discussions repeatedly return to the pain of being treated as lower status after disclosure. People report that ordinary anger becomes “instability,” disagreement becomes “lack of insight,” and ambition becomes unrealistic once a diagnosis is known. Others describe the opposite: employers and families ignored obvious impairment because admitting it would create obligations. The label can lower credibility or provide the first credible account of limitation.

These narratives reveal a feature of power that surveys rarely capture. Mental illness can make a person’s testimony less authoritative precisely when they need to report mistreatment. An abusive partner, employer, or institution can use the diagnosis to reframe conflict as symptom. Yet paranoia, mania, and severe personality disturbance can genuinely distort interpretation. There is no rule that resolves every case. Institutions need procedures that examine evidence without assuming either that diagnosis invalidates testimony or that every accusation is accurate.

The social-defeat account of psychosis remains incomplete. It does not explain why only a minority of excluded people develop psychosis, why specific symptoms emerge, or why risk persists in some supportive environments. It can become a vague metaphor if every disadvantage is relabeled defeat. Genetics, prenatal development, cannabis exposure, neurocognition, and random developmental variation remain central. The value of the hypothesis is not that it replaces biology but that it identifies repeated subordinate status as a plausible component in an interactive model.

The strongest conclusion is relational. Mental health depends partly on whether a person occupies a social world in which they are safe enough to interpret ambiguity, valued enough to expect response, and powerful enough to alter intolerable conditions. Systems create risk when they

concentrate people in positions of chronic threat without credible escape. They sustain illness when they convert symptoms into exclusion and exclusion into further symptoms.

Belonging is not sentimental decoration around psychiatry. It is part of the causal environment. So are rank, recognition, legal standing, and the right to be believed without being declared infallible. A society can injure minds by attacking these conditions. It can also protect minds by making them less scarce.

The Political Mind and the Algorithmic Institution

When Politics Becomes a Psychology

Political systems do not merely distribute money and coercion. They also distribute interpretations. They tell people what counts as success, who deserves humiliation, which losses are personal failures, what kind of person is admirable, which outsiders are dangerous, and whether dissent is a civic act or a betrayal. This is where the language of psychopathology becomes both tempting and dangerous.

Envy is a useful test case. It is easy to turn envy into a political insult. Defenders of inequality accuse egalitarians of wanting to pull the successful down; critics of capitalism accuse consumer societies of manufacturing endless comparison and dissatisfaction. Psychology supports neither slogan in its pure form. Envy is a normal social emotion. It arises when another person's advantage is salient, self-relevant, and painful. Benign envy can motivate effort and learning. Malicious envy can motivate disparagement or a wish to deprive the other person of the advantage. Neither is a mental disorder. Yet systems can amplify the frequency and intensity of the emotion by making rank highly visible, identity dependent on status, and comparison difficult to escape.

This is one reason inequality and social media belong in the same causal vocabulary even though they are not the same phenomenon. A meta-analysis of clinical research found social-comparison evaluations strongly associated with depression and anxiety (McCarthy and Morina, 2020). Research on social networking sites finds recurring links among

upward comparison, envy, and depressive symptoms, but causal direction remains uncertain. In some longitudinal data, depressive symptoms predict later comparison and envy, illustrating the feedback loop. A depressed person searches the environment for evidence of failure; an environment saturated with curated success supplies it. The system need not create the first vulnerability to become an excellent maintenance device.

Demagoguery exploits a related human sensitivity: the desire to convert diffuse discomfort into a legible story. Political rhetoric becomes psychologically powerful when it offers a compact causal map. You are suffering because they humiliated us. Your failures are evidence of betrayal. Complexity is cowardice. The leader sees what the institutions refuse to see. Such narratives can reduce uncertainty and restore status before they change any material condition. Their emotional efficacy is not evidence of delusion. Human beings routinely prefer coherent stories under threat. The danger begins when a system rewards certainty regardless of evidence and makes belonging conditional on repeating the story.

Collective narcissism is one empirically studied form of this dynamic. It is not simply pride in a group. It combines a conviction that the group is exceptional with a grievance that others fail to recognize its greatness. Meta-analytic work links collective narcissism to conspiracy beliefs, especially narratives involving hostile outgroups, and theorists argue that the combination can coordinate undemocratic leaders and constituencies by producing a threatening environment that justifies coercion (Golec de Zavala, Bierwaczek, and Ciesielski, 2022). Other studies find collective narcissism predicts willingness to conspire against members of one's own group. Again, this is not a psychiatric diagnosis. It is a social-psychological disposition that can be politically organized.

Political cults intensify the process by concentrating recognition in a leader. The popular language of "regression to childhood" captures something intuitively recognizable: adults may defer judgment, seek paternal protection, tolerate contradictions, and treat criticism of the leader as an attack on the self. But regression is a psychoanalytic metaphor,

not a reliable population diagnosis. More measurable concepts are available. Authoritarian submission describes willingness to defer to perceived legitimate authority. Identity fusion describes a visceral sense that personal and group identity are one. Leader idealization describes the attribution of exceptional moral or cognitive qualities to a person. Need for cognitive closure captures the attraction of definite answers under uncertainty. Threat sensitivity and motivated reasoning help explain why evidence can be filtered to protect belonging.

Identity fusion is particularly relevant because it predicts costly action, not merely opinion. Across studies of different political causes and national contexts, identity fusion has been a stronger predictor of willingness to fight and die for a cause than sacred values or moral conviction considered alongside it (Martel et al., 2021). Research on violent extremism also emphasizes that fusion interacts with perceived threat, social networks, and moralized beliefs. Most fused people do not become violent, and most people with extreme overvalued beliefs do not become attackers. The scientific point is conditional: systems can create environments in which fusion is rewarded, threat is rehearsed, and self-sacrifice becomes morally legible.

This distinction prevents two opposite errors. The first is psychiatric exculpation: if perpetrators are called "mad," institutions disappear from the explanation. Bureaucrats, propagandists, informants, guards, and voters can participate in cruelty while remaining psychiatrically ordinary. The second is psychiatric innocence: because political behavior is not a disorder, it has no mental-health dimension. In fact, the same system can organize harmful behavior among psychologically intact members while producing trauma, anxiety, depression, paranoia, and substance misuse among targets and dissenters.

Paranoia requires similar discipline. Clinical paranoia involves persistent, unjustified beliefs of persecution that impair functioning. Political suspicion may be rational when governments actually surveil, employers retaliate, or police persecute. An East German dissident who

suspected mail interception was not necessarily delusional. The institution can make a paranoid style adaptive by making hidden punishment real. After the system changes, the learned vigilance may persist and become costly. Political repression studies show exactly this kind of long tail: former prisoners and persecuted citizens report anxiety, hyperarousal, depressive symptoms, PTSD, and dissociation long after the immediate danger ends.

Conspiracy belief occupies the boundary. Some conspiracy theories are false; some conspiracies are real. Believing a false conspiracy is not by itself a mental illness. Population research generally treats conspiracy beliefs as a continuum associated with mistrust, threat perception, identity, and epistemic style. A system that repeatedly lies can make generalized suspicion more rational. A system that saturates media with unfalsifiable accusations can make evidence irrelevant. Both degrade what might be called epistemic safety - the ability to form beliefs in an environment where institutions are fallible but corrigible.

Epistemic safety matters to mental health because uncertainty is not purely intellectual. If a person cannot tell whether a boss is joking or threatening, whether a benefit will arrive, whether a neighbor is an informant, whether a news report is propaganda, or whether a diagnostic label will be used against them, attention remains mobilized. Chronic ambiguity plus high stakes is psychologically expensive. Totalitarian systems often create this condition deliberately; dysfunctional organizations can create it accidentally.

Political humiliation is another bridge between social pathology and clinical suffering. Humiliation differs from simple loss because it contains an audience and a status message: you have been lowered and others can see it. Systems can institutionalize humiliation through public denunciation, caste ritual, racial segregation, bureaucratic degradation, workplace bullying, or algorithmic pile-ons. Humiliation can provoke shame, rage, withdrawal, revenge fantasies, depression, and trauma-like symptoms. None of these responses proves disorder. The key question is persistence

and impairment. A social emotion becomes a clinical problem when the person's regulatory system remains trapped after the event or the humiliating environment continues.

Demagoguery often converts humiliation into externalized grievance. This can be psychologically relieving. A person who felt individually defective may feel restored by a narrative in which the group is noble and cheated. The relief can coexist with deteriorating reality testing about politics. But "reality testing" must be used carefully outside the clinic. Political error is not psychosis. Millions of people can believe false claims through conformity, media ecosystems, identity, and motivated reasoning without having a psychotic disorder. Calling political opponents mentally ill is both scientifically weak and historically dangerous.

The more useful concept is social pathology. A social process is pathological in a functional sense when it reliably damages the capacities a system claims to serve: deliberation, cooperation, safety, truth-seeking, or human development. Demagoguery can be socially pathological because it rewards false simplification and turns correction into disloyalty. Malicious envy can be socially pathological when institutions reward sabotage over improvement. A personality cult can be socially pathological when feedback to leaders disappears. These are not DSM diagnoses. The term is analogical, and the book should say so every time the distinction matters.

There is nevertheless a continuum from social pathology to psychiatric burden. A movement organized around permanent emergency may keep members anxious and hypervigilant. A purity culture can produce obsessive monitoring of thought and speech without causing obsessive-compulsive disorder. A leader-centered movement can produce dependency and fear of exclusion without causing dependent personality disorder. A conspiracy ecosystem can intensify suspiciousness without causing delusional disorder. Clinical labels should not be stretched to fit politics. Mechanisms can be shared without diagnoses being identical.

This is also where the concept of moral injury becomes useful. Moral injury was developed largely in military contexts to describe lasting distress

after perpetrating, failing to prevent, or witnessing acts that violate deeply held moral beliefs. Political systems can create analogous situations. A civil servant falsifies a record to protect a superior. A teacher denounces a colleague. A soldier participates in abuse. A party member remains silent while a neighbor disappears. The psychological consequences may include guilt, shame, anger, spiritual crisis, depression, and self-destructive behavior. The causal event is social and moral; the symptoms are individual and biological.

Envy, demagoguery, and cultic politics therefore belong in a book about mental illness, but only after the conceptual border is fortified. They belong because they can shape risk environments, maintain maladaptive cognition, recruit normal motives into destructive feedback loops, and sometimes precipitate clinical suffering. They do not belong as excuses to diagnose populations.

The strongest statement is not that bad political systems make their supporters insane. It is that political systems can train attention, emotion, identity, and expectation. They can make some forms of vigilance rational, some forms of cruelty rewarded, some forms of dependency honorable, and some forms of doubt dangerous. When the environment changes, the trained mind may not change at the same speed.

This returns us to the core thesis of *The Illness Machine*. A system becomes psychologically powerful when it can repeatedly select the same response. It need not insert an idea directly into a brain. It can change the payoff structure around fear, belonging, comparison, and truth. Over time, what began as adaptation can become character, habit, symptom, or institution.

The Algorithmic Institution

The old asylum controlled a person's space. The modern platform controls a sequence of attention. It does not lock the door. It learns which door a person is least able to stop opening.

This comparison should not be exaggerated. Social media connects families, sustains minority communities, distributes knowledge, enables political organization, and gives isolated people access to peers they could never find locally. A feed is not a psychiatric ward, and a recommendation algorithm is not a doctor. Yet platforms have become social institutions in a strict sense. They allocate visibility, status, punishment, belonging, and information at enormous scale. Their rules are privately written, continuously altered, and enforced through ranking systems that most users cannot inspect.

The public debate often asks whether “screen time” causes depression. The variable is too crude. Time on a device may involve friendship, harassment, creative work, pornography, political conflict, school, sleep-displacing entertainment, or support during crisis. Treating all of it as one exposure resembles studying nutrition by counting minutes spent eating. Reviews therefore find a mixed literature. An umbrella review of 25 reviews concluded that most characterized associations between adolescent social-media use and mental health as weak or inconsistent, while a minority judged them substantial and harmful (Valkenburg et al., 2022). Large correlational studies often find small average associations, sensitive to measurement and analytic choices (Orben and Przybylski, 2019).

Small average effects do not settle the question. An exposure used by billions can matter at population scale even if the mean shift is modest. More importantly, averages can combine people who benefit, people who are unaffected, and a vulnerable minority who are harmed. Experience-sampling research has found substantial person-specific variation: the same pattern of use is associated with improved well-being for some adolescents and worse well-being for others (Beyens et al., 2020). The relevant causal unit may be an interaction among user, content, timing, social position, and design.

Causal evidence is stronger than it was a decade ago. Luca Braghieri, Ro’ee Levy, and Alexey Makarin exploited Facebook’s staggered introduction across American colleges between 2004 and 2006.

Comparing mental-health survey responses before and after access arrived at different campuses, they found that Facebook's rollout worsened several indicators of student mental health and increased the likelihood that poor mental health impaired academic performance. Additional analyses pointed toward unfavorable social comparison as a mechanism (Braghieri et al., 2022). The design is quasi-experimental rather than randomized, and early Facebook differs from today's platforms, but the result challenges the claim that all associations reflect distressed people simply using social media more.

A large randomized experiment before the 2018 United States midterm election paid participants to deactivate Facebook for four weeks. Deactivation increased offline activity and subjective well-being, reduced political knowledge and polarization, and led to lower Facebook use after the experiment (Allcott et al., 2020). The effect was not a simple liberation into community; some extra time went to television and solitary activity. Removing one platform changed a bundle of behavior, not only mood.

More recent intervention studies remain mixed in design and strength. A 2025 cohort study of young adults used objective phone data and an optional one-week reduction period. Participants who chose the "detox" showed reductions in self-reported depression, anxiety, and insomnia, but the lack of randomized assignment makes expectation, self-selection, and regression to the mean plausible alternatives (Calvert et al., 2025). Such findings are suggestive, not proof that a week away from social media is treatment.

The mechanism matters because policy aimed at total duration may miss the harmful parts. Social comparison is one pathway. Platforms display unusually curated bodies, relationships, wealth, productivity, and moral performance. Users compare an unedited interior with selected exteriors. The comparison is intensified by metrics that translate social response into visible counts. Status, once inferred from a small group, becomes numerically refreshed throughout the day.

Harassment is another pathway. Bullying that once ended at the school gate can become persistent, searchable, and witnessed by a large audience. The target loses temporal and spatial refuge. Viral humiliation compresses social defeat into an event that can be replayed indefinitely. Again, the platform does not invent human cruelty, but it alters its scale, persistence, and reward structure.

Sleep provides a more mundane route. Notifications, autoplay, endless feeds, nighttime light, emotional arousal, and fear of missing out delay sleep. Sleep loss worsens mood regulation, attention, anxiety, and vulnerability to mania or psychosis. A system need not produce a new psychiatric mechanism if it reliably amplifies an old one.

Compulsive use introduces the problem of agency. Platforms are valuable partly because people want to use them. Yet preference at the moment of a click may conflict with longer-term preference. Research on digital addiction and willingness to pay suggests that self-control problems account for part of usage and that users may value tools that help them reduce it. This does not make every user addicted. It shows that “they chose it” is an incomplete defense when a product is optimized to exploit predictable failures of self-regulation.

Recommendation systems also shape symptom content. A person who searches once for panic, dissociation, autism, eating disorders, or conspiratorial explanations may receive a dense stream of related material. The system infers interest, not truth or clinical need. Repetition makes a framework cognitively available and socially validated. In benign cases, the user discovers language and support. In dangerous cases, the feed narrows interpretation around illness, grievance, self-harm, extreme dieting, or persecution.

This is not equivalent to brainwashing. Users actively select, reject, and reinterpret content. Algorithms respond to behavior, so causality is reciprocal. A distressed person clicks on distress-related material; the platform supplies more; the supplied material changes attention and

expectation; those changes produce more clicks. The result is a feedback loop in which selection and causation become inseparable.

Online self-diagnosis belongs inside this loop. Qualitative research on Reddit finds an unresolved divide. Participants describe self-diagnosis as validating, access-enabling, and sometimes necessary when professional services are expensive, biased, or unavailable. Others describe it as inaccurate, identity-forming, or harmful. The recurring fear is that a provisional explanation can become self-fulfilling, while widespread suspicion of self-diagnosis makes people with genuine problems less likely to be believed (Underhill and Foulkes, 2025).

The platform profits from both certainty and conflict. “You may have several possible explanations and need a careful assessment” is clinically responsible and algorithmically weak. “Seven signs you were secretly traumatized” offers recognition, surprise, and shareability. Diagnostic language becomes content because it converts diffuse discomfort into a story with a named protagonist: the self.

Mental-health discourse can then produce a paradox. Greater literacy reduces shame and encourages help-seeking, but constant symptom education can increase monitoring of ordinary fluctuations. Young people learn to describe discomfort with sophisticated clinical vocabulary before learning which discomfort is transient, which is contextual, and which requires treatment. A culture that once denied suffering may swing toward interpreting every difficulty through pathology. The system alternates between invalidation and overrecognition.

Commercial mental-health technology adds another institutional layer. Apps offer screening, coaching, meditation, mood tracking, and chatbot conversation. Some expand access and help users. Others convert distress into data, retention, and subscription revenue. The central risk is not merely that an algorithm gives a wrong answer. It is that the business model may reward continued engagement more reliably than recovery. A service that becomes less necessary when the user improves must be designed against its own economic temptation.

Artificial-intelligence companions intensify this issue. As of 2026, evidence about long-term effects on loneliness, dependency, delusion, and help-seeking remains limited and rapidly changing. Case reports and early studies justify caution, especially for users in psychosis or crisis, but they do not support claims that chatbots generally create mental illness. The plausible danger is a personalized feedback partner that never tires, rarely challenges enough, and can reinforce an interpretation because agreement sustains the interaction. The plausible benefit is immediate, nonjudgmental support and practical guidance. Both possibilities require longitudinal and adversarial evaluation rather than marketing anecdotes.

The old institution recorded a patient in a chart. The algorithmic institution records nearly everyone. Search history, scrolling, sleep timing, location, social response, and language can be used to infer distress. Early detection may allow help before crisis. The same inference can affect insurance, employment, advertising, policing, or credit. A system that predicts vulnerability gains power over the vulnerable even when the prediction is accurate.

Mental-health surveillance therefore cannot be evaluated only by predictive performance. It requires limits on purpose, consent, retention, appeal, and downstream use. A teenager may consent to a wellness app without consenting to a durable psychiatric profile. A worker may answer an anonymous survey that becomes identifiable through data linkage. Prevention can become preemption, in which institutions act on a predicted disorder before the person has any opportunity to contest the model.

Public internet discussions reveal a deep ambivalence toward these systems. People credit online communities with saving their lives and blame the same platforms for worsening comparison, compulsive use, self-harm exposure, and diagnostic obsession. Clinicians describe patients arriving with useful self-observation and rigid scripts learned from influencers. Users distrust corporate therapy apps yet disclose intimate material to general-purpose chatbots. The contradiction is rational. The

systems are simultaneously support networks, markets, laboratories, and mirrors.

A simplistic anti-technology conclusion would repeat the errors of earlier moral panics. Books, radio, television, games, and the internet have each been accused of corrupting minds. Average effects are often smaller than headlines suggest, and restriction can harm people whose online life is their main source of belonging. The scientific challenge is to identify specific design features, vulnerable periods, and user-level heterogeneity.

The relevant unit of responsibility is not only the user's choice. Platforms choose defaults, ranking objectives, notification timing, friction, metrics, moderation, and the accessibility of exit. Regulators choose whether internal data remain private and whether independent researchers can evaluate harm. Schools and families choose how digital life is integrated with sleep, movement, and face-to-face community. The system is distributed, which is why responsibility so easily evaporates.

An algorithmic institution creates mental-health risk when it repeatedly converts vulnerability into engagement, comparison into status data, uncertainty into identity, and attention into a resource extracted without effective limits. It can reduce risk when it supports connection, gives users control, interrupts harmful spirals, protects sleep, and directs acute cases toward competent human care.

The question is not whether the phone makes us sick. It is whether systems that learn our vulnerabilities are obligated to do more than monetize the accuracy of what they learn.

The Case Against This Book

Imagine a pair of identical twins. They began as one fertilized egg and therefore carry nearly the same inherited sequence. One develops schizophrenia. The other does not.

The pair is not an argument against biology. It is one of the strongest demonstrations that biology matters. Identical twins resemble one another in risk far more than fraternal twins or ordinary siblings. In a nationwide Danish study of more than thirty-one thousand twin pairs, the estimated heritability of schizophrenia was 79 percent. Yet the concordance rate among identical twins was only 33 percent. Most identical co-twins of a person with schizophrenia did not receive the same diagnosis during the study period (Hilker et al., 2018). The numbers refuse both ideologies at once. Genetic liability is powerful. Genetic liability is not a verdict.

A book about social causation must spend time with the evidence least convenient to it. Otherwise it becomes a prosecution brief rather than an inquiry. The strongest objection is not that social environments are irrelevant. Almost nobody serious believes that war, abuse, isolation, or deprivation have no psychological effects. The objection is that the phrase society creates mental illness can claim far more than the evidence permits. It can confuse population risk with individual explanation, ignore reverse causation, treat every diagnosis as social control, and replace one reductionism with another.

Heritability is the first discipline. Across thousands of twin studies and a vast range of human traits, genetic influences are substantial rather than exceptional (Polderman et al., 2015). Psychiatric disorders aggregate in families. Schizophrenia, bipolar disorder, autism,

attention-deficit/hyperactivity disorder, major depression, and substance dependence all show inherited components, although estimates differ and diagnostic boundaries overlap. Contemporary genomic research finds not one “gene for” most disorders but many variants of individually tiny effect, along with rarer variants that can carry larger risks. The genome does not write a diagnosis in plain language. It changes probabilities across development.

This matters because social explanations can become morally flattering. It is satisfying to discover that suffering originates in an unjust system rather than in the body. The story supplies an antagonist, converts private shame into public critique, and suggests that reform could prevent recurrence. Sometimes that is exactly right. Sometimes the desire for a morally legible cause outruns the data. A person can develop severe obsessive-compulsive disorder in a safe family, bipolar mania during a period of success, or psychosis without a history that yields a persuasive political explanation. Nature does not owe us causes that match our ethics.

Heritability is also frequently misunderstood in the opposite direction. An estimate of 79 percent does not mean that 79 percent of one person's schizophrenia is genetic and 21 percent is environmental. It describes variation in liability within a particular population under particular conditions. If every child in a population receives equally poor nutrition, genetic differences may explain much of the remaining variation in height even though nutrition profoundly determines average height. Change the environment and the estimate can change. A highly heritable condition can still be preventable in part, just as phenylketonuria is genetically caused but its most severe consequences can be prevented through diet.

The identical-twin result therefore leaves a large scientific territory between “genetic” and “social.” Development contains prenatal events, infection, obstetric complications, substances, random cellular processes, differences in gene expression, and experiences not shared even by children raised together. The environment in behavior genetics is not synonymous with politics. It includes everything not attributed to the measured genetic

component, including measurement error. Social theory cannot simply annex the unexplained remainder.

The second objection is social selection. Mental illness can produce the social conditions later blamed for maintaining it. Depression impairs concentration, energy, confidence, and attendance. Mania can destroy employment, savings, and relationships. Psychosis can interrupt education and make ordinary institutional encounters difficult. Addiction can become both adaptation and engine, relieving distress briefly while generating debt, conflict, legal exposure, and physical dependence. Families may spend resources on care, reduce working hours, or fracture under pressure. The causal arrow from illness to disadvantage is not an excuse invented by defenders of inequality. It is visible in biographies and longitudinal data.

Classic research on socioeconomic status and psychiatric disorder found different balances for different conditions. In a 1992 *Science* study designed around the causation-selection problem, Bruce Dohrenwend and colleagues concluded that social selection appeared more important for schizophrenia, while social causation appeared more important for some depression, antisocial personality, and substance-use outcomes in the population studied (Dohrenwend et al., 1992). Later registry work has continued to find evidence that familial liability contributes to downward social drift in schizophrenia (Sariaslan et al., 2016). These findings do not abolish childhood adversity, migration, urbanicity, or discrimination as possible causes of psychosis. They show why an observed concentration of psychosis in poor neighborhoods cannot be read as a simple environmental experiment.

The most realistic model is often reciprocal. A vulnerability produces an early failure at school. The failure lowers status and opportunity. The resulting humiliation and insecurity increase symptoms. Symptoms damage relationships and employment. Exclusion then removes protective routines and exposes the person to danger. The system did not necessarily

initiate the loop, but it can accelerate it. Causation and selection become alternating moves rather than rival doctrines.

The third objection comes from treatment. If antipsychotic medication prevents relapse, if antidepressants outperform placebo on average, and if structured psychotherapies help many patients, then mental disorders cannot be dismissed as names for political discomfort. A large meta-analysis of maintenance treatment in schizophrenia found that antipsychotic drugs reduced one-year relapse from 64 percent on placebo to 27 percent on medication, while also producing important adverse effects and substantial variation in response (Leucht et al., 2012). A network meta-analysis of 522 antidepressant trials involving more than 116,000 adults found all twenty-one studied drugs more efficacious than placebo for acute major depression, although effect sizes, acceptability, bias, and certainty varied (Cipriani et al., 2018). Hundreds of randomized psychotherapy trials likewise show that cognitive-behavioral and other bona fide therapies can improve depression, without implying that one school has discovered the single true mechanism of despair (Cuijpers et al., 2021; Cuijpers et al., 2023).

These results are not a referendum on whether the causes are biological or social. Treatments can act downstream. An infection may be socially distributed and still require antibiotics. A panic disorder precipitated by an abusive workplace may respond to cognitive-behavioral therapy after the person has left. Antipsychotics can reduce relapse even when social defeat, cannabis exposure, or developmental adversity contributed to onset. Efficacy establishes that the targetable process is real; it does not reveal the complete history that produced it.

Still, the treatment evidence sets boundaries. Social reform should not become a reason to withhold care while waiting for justice. A person in acute mania does not need to hear that capitalism is dysregulating. A person with terrifying voices should not be recruited into a theory that romanticizes psychosis as insight. The record of psychiatry includes coercion and diagnostic abuse, but the record of anti-psychiatric

romanticism includes abandonment. It is possible to oppose degrading institutions and still believe that severe mental illness can destroy agency from within.

The fourth objection is diagnostic heterogeneity. “Mental illness” is too broad to serve as one outcome. The Cherokee income supplement improved some childhood behavioral symptoms but not anxiety or depression. Neighborhood relocation produced different effects across gender and developmental stage. Cash transfers usually yield modest average improvements, not psychiatric transformation. Anti-bullying programs reduce bullying and some associated distress without eliminating childhood disorder. Housing First reliably improves housing stability, yet psychiatric symptoms and substance use do not always improve to the same degree. Social interventions often work where their mechanism reaches and fail where it does not.

This unevenness is not a technical inconvenience. It is evidence about the world. A universal social theory predicts too much. If one reform is expected to improve every diagnosis, nonresponse will be explained away as insufficient dosage of justice. Science requires the opposite habit. When effects are small, absent, delayed, or concentrated in one subgroup, the theory must contract.

The fifth objection concerns replication and seductively specific mechanisms. In 2003, a celebrated study reported that a variant in the serotonin-transporter gene moderated the effect of stressful life events on depression (Caspi et al., 2003). The result offered an elegant picture: the same adversity could produce different outcomes depending on genotype. It became one of the emblematic findings of gene-environment interaction. Larger collaborative analyses later found no evidence for a strong interaction of the originally proposed form (Culverhouse et al., 2018).

The lesson is not that genes and stress never interact. They must interact in the ordinary sense that organisms with inherited characteristics respond to environments. The lesson is that identifying one reproducible

molecular switch is much harder than drawing a plausible diagram. Social psychiatry is vulnerable to the same temptation as biological psychiatry: a complex pattern is converted into a clean mechanism before the evidence has earned the simplicity.

A related danger appears in neuroscience language. Social rejection “lights up” a brain region; chronic stress “rewires” the brain; poverty “gets under the skin.” These phrases can communicate genuine biological embedding, but they can also function as ceremonial validation. Suffering seems to become real only after a scanner detects it. Then an uncertain group-level association is narrated as an irreversible injury inside every exposed person. The political message gains authority while the science loses precision.

The sixth objection is resilience. Most people exposed to any common social risk do not develop the specific disorder under discussion. Most people who lose jobs do not develop major depression. Most bullied children do not become psychotic. Most migrants do not experience psychosis. Most social-media users do not become suicidal. Human beings adapt, find allies, reinterpret events, leave environments, and possess biological differences that alter susceptibility. A theory centered only on harm can make resilience look like inconvenient noise.

Yet resilience should not be used as an acquittal for systems. Most smokers do not die of lung cancer, and most drivers in dangerous intersections survive. Causation is compatible with variable response. The scientific task is to identify who is vulnerable, through which mechanism, under what duration and intensity of exposure, and with which buffers. The moral task is harder. A society cannot wait for perfect individualized prediction before reducing an exposure that creates preventable risk.

The seventh objection is political. Once mental health becomes an objective of every institution, governments and employers acquire a new language for governing. A school can justify surveillance as early detection. A company can call intrusive monitoring a well-being program. A city can suppress protest because conflict is “traumatizing.” A platform can collect

intimate data in the name of prevention. The history of psychiatry warns that health language expands authority as readily as it relieves suffering.

A mental-health impact assessment may be valuable, but it cannot become a warrant to engineer citizens into permanent calm. Anxiety can signal danger. Anger can expose injustice. Grief can honor attachment. Dissent can be sane in a sick institution. The aim of social prevention is not a population that never experiences negative emotion. It is a population less needlessly exposed to conditions that generate disabling and persistent suffering.

The strongest case against this book therefore succeeds against its most dramatic possible title. No coherent evidence shows that social systems manufacture all mental illness. Some disorders have substantial inherited and neurodevelopmental components. Illness itself produces disadvantage. Clinical treatments have genuine effects. Social interventions are heterogeneous. Many exposures are neither necessary nor sufficient, and many individual cases cannot be explained from social history alone.

But the countercase does not defeat the qualified argument. It improves it. A system can be one cause among several. It can determine whether liability is activated, whether symptoms are interpreted humanely, whether treatment is available, and whether recovery is materially possible. It can initiate a loop or enter after onset and become its maintenance machinery. The fact that an intervention does not cure every patient is not evidence that the exposure was harmless.

The question mark in *Sick by Design?* survives. The answer is not that every disorder was designed. It is that societies repeatedly design distributions of risk and then describe the resulting suffering as if it arrived from nowhere.

Repairing the Causal Environment

A patient describes panic that began after a supervisor started changing shifts without notice, humiliating workers in meetings, and threatening dismissal by text late at night. The clinic can teach breathing, examine catastrophic thoughts, prescribe medication, and write a medical certificate. Each response may help. None changes the supervisor.

This is the practical scandal at the center of social psychiatry. The person arrives where treatment exists, while the cause may sit in a payroll system, a tenancy contract, a school corridor, an immigration office, a family hierarchy, or a recommendation algorithm. Clinical care is asked to repair downstream what other institutions continue to manufacture upstream. When therapy succeeds, the institution receives a healthier worker, tenant, student, or user without having to alter the condition that caused the damage.

The opposite mistake is equally dangerous. A clinician cannot suspend care until the workplace reforms. Social explanation without immediate relief can become an elegant form of neglect. Repair therefore has to operate on more than one timescale. The person needs help surviving the present. The environment needs to stop reproducing the same injury. Public policy needs to reduce the number of people who reach the clinic through that route.

The Bucharest Early Intervention Project offers the clearest image of environmental repair because its intervention was not a lesson in coping with deprivation. Children raised in profoundly deprived institutions were placed, by random assignment, into specially supported foster families or

remained in care as usual. Foster care improved several developmental and psychiatric outcomes, especially when placement occurred earlier, although it did not erase every consequence of institutional rearing (Zeanah et al., 2009; Wade et al., 2018; King et al., 2023). The intervention changed the child's daily social world: the availability of an attachment figure, the contingency of response, the continuity of care, and the possibility of being known as one child rather than managed as one unit.

That distinction matters. Many mental-health programs teach individuals to become more resilient inside unchanged systems. Resilience training can be useful, but it can also function as institutional laundering. A hospital reduces staffing, a company increases unpredictability, a school tolerates bullying, and the affected people receive a seminar on emotional regulation. The intervention asks the nervous system to absorb a cost that the organization has chosen not to pay.

Organizational repair begins by asking whether the institution controls a relevant exposure. Schools cannot eliminate every childhood vulnerability, but they control supervision, reporting, sanctions, classroom norms, and the social rewards attached to cruelty. The KiVa anti-bullying program treats bullying as a group process rather than a private conflict between one aggressor and one victim. A large multicenter cluster-randomized trial in British primary schools found reductions in victimization with effects judged small to moderate but meaningful at population scale (Bowes et al., 2024). The lesson is not that one branded program solves bullying. It is that the audience, rules, and reinforcement structure around aggression can be changed.

Workplaces likewise control more than access to an employee-assistance telephone line. They determine workload, autonomy, scheduling, job security, role clarity, procedural fairness, and whether reporting abuse is safe. Research on job strain consistently implicates high demands combined with low control, while evidence for many organization-level mental-health interventions remains less mature than the evidence for describing the problem. This gap is politically convenient. It is much easier

to randomize an app than a management hierarchy. The scarcity of trials should create research urgency, not an assumption that structural changes are ineffective.

A credible workplace intervention would not promise happiness. It would reduce chronic unpredictability, provide real control where possible, protect recovery time, make workloads compatible with human attention, and create appeal mechanisms that do not depend on the goodwill of the person being challenged. The relevant outcome would not be only a short well-being questionnaire. It would include sickness absence, turnover, sleep, depressive symptoms, harassment reports, medication use, and whether gains persist after publicity around the program has faded.

Economic policy provides more direct experimental leverage. Cash-transfer research shows small average improvements in mental health and subjective well-being across low- and middle-income settings, with larger transfers tending to produce larger effects (McGuire et al., 2022). The small mean should not be inflated into a cure, but neither should it be dismissed. A modest effect distributed across more than one hundred thousand participants can represent substantial prevented suffering. Cash also improves outcomes not captured by symptom scales: food, school participation, healthcare access, autonomy, and the capacity to survive a shock.

The mechanism is rarely “money makes people happy.” Income changes the frequency of impossible choices. It can reduce conflict over bills, create distance from an abusive partner, prevent a utility shutoff, pay for transport to treatment, or allow a parent to be present. Debt relief may free attention otherwise consumed by threat. Minimum-wage policy can alter the background risk of despair. Social insurance can make unemployment a difficult transition rather than a fall through the floor. These policies are mental-health interventions without becoming psychiatric programs.

Their design still matters. Benefits that are unpredictable, humiliating, difficult to claim, or withdrawn abruptly can create new stress.

Conditionality can support useful behavior in one context and become coercive surveillance in another. A payment too small to change the constraint may produce a statistically detectable effect while leaving the causal environment intact. Recipients in qualitative studies often describe cash as necessary but insufficient and report that agency depends on how the program is administered, not only on the amount transferred (Yoshino et al., 2023).

Housing makes the boundary between social and clinical care almost impossible to defend. Stable housing can reduce exposure to violence, sleep disruption, theft, police contact, infection, and the daily cognitive work of survival. Housing First programs reverse the traditional sequence in which people must demonstrate sobriety or treatment compliance before receiving a home. Randomized evidence from Canada found major improvements in housing stability for homeless adults with mental illness, while effects on psychiatric symptoms and substance use were more variable (Aubry et al., 2015). A home is not an antipsychotic. It is the platform on which treatment, routine, privacy, and recovery become more possible.

Neighborhood interventions reveal why even sensible structural changes can have uneven outcomes. Moving to Opportunity offered randomly selected families vouchers to move from high-poverty neighborhoods. Some mental-health outcomes improved, especially for girls, while some outcomes for boys worsened. Disrupted peer networks, new status comparisons, policing, school culture, and the social meaning of relocation may have mattered (Leventhal and Brooks-Gunn, 2003; Osypuk et al., 2012). Moving an address without rebuilding belonging can exchange one exposure for another.

This finding should inoculate reformers against architectural fantasy. A neighborhood is not merely an index of poverty. It is a network of relationships, threats, reputations, services, opportunities, and informal protection. Social interventions fail when they move resources but ignore meaning. The same is true of de-institutionalization. Closing a degrading

hospital is not liberation if the person is discharged into homelessness, jail, or isolation. Rights require material infrastructure.

Repair after illness has begun must also restore social roles. Employment is not automatically therapeutic; bad work can be pathogenic. Yet wanted, supported employment can provide income, rhythm, identity, competence, and relationships. Individual Placement and Support rejects the assumption that people with severe mental illness must spend years proving readiness in sheltered settings. It helps them seek competitive employment rapidly while integrating vocational and clinical support. Meta-analyses show substantially better employment outcomes than traditional vocational services, including among young adults with serious mental illness or early psychosis (Bond et al., 2023). The intervention treats participation as part of recovery rather than a prize awarded after recovery is complete.

Families belong in the same causal environment, but history demands caution. Earlier psychiatric theories sometimes blamed mothers for schizophrenia and autism with little evidence and enormous cruelty. Contemporary family interventions do something different when practiced well. They do not identify a guilty relative. They reduce conflict, improve communication, teach recognition of relapse, and help families manage fear and burden. A network meta-analysis of ninety randomized trials found that most family-intervention models reduced relapse in schizophrenia compared with usual treatment, although certainty varied and later reviews have stressed limitations in parts of the evidence base (Rodolico et al., 2022; Chien et al., 2024). The family is neither the hidden cause of every disorder nor irrelevant to its course.

Institutional care can be repaired through procedural justice. People often remember not only whether they were admitted involuntarily but whether they were heard, informed, humiliated, threatened, restrained, or given a credible route to regain control. Acute danger can make coercion unavoidable in some cases. That necessity does not make every method equivalent. Advance directives, independent advocacy, de-escalation,

transparent review, continuity with trusted clinicians, and post-incident accountability can reduce the experience of arbitrary power. The ethical target is not a fictional world with no difficult decisions. It is a system in which necessity cannot become habit.

Clinical formulation also changes when the social world is treated as causal rather than decorative. The assessment still asks about symptoms, duration, impairment, family history, substances, sleep, physical illness, and risk. It also asks what changed before the symptoms began, which threats remain active, where the person has control, who benefits from the current arrangement, and what material constraint prevents recovery. These questions do not replace diagnosis. They prevent diagnosis from becoming the end of explanation.

A socially informed clinician may write two treatment plans at once. One targets panic, mood, voices, compulsions, or sleep. The other targets eviction, violence, debt, isolation, discrimination, or work. The second plan requires partnerships with housing services, legal aid, schools, employers, peer organizations, and social protection. Such coordination is slower than prescribing and difficult to reimburse. That is an institutional design problem, not evidence that the needs are nonmedical.

“Social prescribing,” in which clinicians connect patients to community groups, exercise, arts, volunteering, or practical support, expresses the right instinct but can become another slogan. A referral cannot create a community organization that lacks funding. Volunteering is not a substitute for income. A walking group cannot neutralize domestic violence. Social interventions deserve the same demands for mechanism, comparison, adverse-effect monitoring, and long-term outcomes as clinical treatments.

The emerging algorithmic institution requires its own repair logic. Platforms possess experimental capacity on a scale public-health researchers rarely have. They can test whether removing visible popularity counts changes comparison, whether nighttime friction protects sleep, whether recommendation limits interrupt self-harm spirals, and whether

users benefit from control over the objectives shaping their feeds. Yet companies usually control the data, define the outcomes, and decide which results become public. A science of platform mental health is impossible when the institution producing the exposure monopolizes the evidence.

Independent data access, preregistered safety evaluations, age-sensitive defaults, meaningful exit, and limits on mental-health inference should be treated as infrastructure rather than corporate generosity. The standard cannot be that a feature is safe until outsiders somehow prove harm without access to the system. Nor should regulation assume that all engagement is pathological. The goal is to expose and constrain design choices that predictably convert vulnerability into retention.

Artificial-intelligence systems add a requirement that older media never faced: the system can adapt its response to the individual's disclosed fears, beliefs, and dependencies. A responsible mental-health agent must know when conversational fluency exceeds evidence, when agreement can reinforce delusion, when escalation to human care is needed, and when continued interaction serves the provider more than the user. Safety cannot be reduced to refusing certain phrases. It includes memory boundaries, auditability, uncertainty, crisis protocols, and evaluation over months rather than one conversation.

At the policy level, a mental-health impact assessment could make causal responsibility visible. Major reforms in housing, welfare, labor, education, migration, policing, and digital regulation would be evaluated for plausible effects on chronic threat, control, belonging, sleep, humiliation, and access to care. The assessment would not assign one numerical "mental-health score," because false precision invites gaming. It would state mechanisms, affected groups, uncertainty, evidence strength, and indicators that should trigger revision.

Such an instrument carries dangers. Governments could use well-being to suppress legitimate risk-taking, protest, or cultural difference. Experts could privilege measurable calm over autonomy. Employers could select people predicted to remain stable rather than improving work. Safeguards

therefore have to include rights, transparency, appeal, and representation by those exposed to the policy. Mental health is a public value, not a license for benevolent total administration.

The deepest reform is conceptual. Prevention should not mean manufacturing a society without sorrow. Loss, conflict, fear, dependence, and mortality cannot be designed away. The aim is narrower and more defensible: reduce involuntary, prolonged, and unequally distributed exposures that make disabling mental disorder more likely; protect routes out of harmful systems; and provide effective care when prevention fails.

A good mental-health system helps people adapt to reality. A good society also asks whether the reality demanding adaptation is needlessly cruel.

Society Is Part of the Treatment

A society does not have a mind. It cannot be diagnosed with depression, psychosis, or a personality disorder. Describing a culture as narcissistic, a bureaucracy as paranoid, or an economy as insane is rhetorically satisfying and scientifically careless. Systems do not suffer. People do.

Yet systems organize the conditions under which people suffer. They distribute exposure to violence and the means of escape. They decide whether unemployment becomes temporary disruption or total loss of identity and health care. They decide whether a child's caregiver has time, whether a tenant can sleep, whether a refugee can plan, whether a diagnosis opens a door or closes the rest. They determine whether care is a relationship or a sequence of eligibility tests.

The evidence in this book supports a qualified yes to its central question. Severe deprivation can causally alter development. Violence and war generate disorders. Poverty and mental illness form reciprocal loops, and experimentally or quasi-experimentally changing income sometimes changes symptoms. Work conditions, housing, neighborhood, discrimination, and structural stigma affect risk and course. Institutions can amplify withdrawal, dependency, trauma, and hopelessness. Labels can reduce confusion while creating stigma. Digital systems can connect and destabilize, with effects hidden by averages.

The evidence also supports a firm no to the total theory. Society does not explain every mental disorder. Genetic and biological liabilities are substantial. Symptoms can arise in safe and loving environments. Social

interventions often have small or mixed clinical effects. The individual is not clay, and the system is not an omnipotent sculptor.

The argument that survives is relational. Mental illness emerges from causal networks that cross bodies, biographies, relationships, and institutions. The boundaries between these levels are administratively convenient and scientifically porous. A social cause becomes cortisol, sleep loss, inflammation, learning, vigilance, and despair. A biological vulnerability becomes school failure, job loss, family conflict, stigma, and poverty. The loop is the object.

This is why the question “Is it biological or social?” has outlived its usefulness. It asks us to select one location from a circuit. The better questions are where the loop began, which links are strongest, which are changeable, and which intervention carries the least harm. Sometimes the answer is lithium. Sometimes it is a locked door opened safely. Sometimes it is money, housing, sleep, legal protection, a different job, a foster family, or a person who remains present. Often it is several at once.

The most tragic systems are not always those that create the first symptom. They are those that convert a temporary crisis into a social destiny. A person loses work, housing, relationships, credibility, and access to ordinary roles, then the resulting disability is cited as proof that none of those things could have been preserved. Chronicity becomes self-confirming.

The reverse is also possible. A system can convert vulnerability into a survivable difference. It can provide early care without fatalism, treatment without humiliation, income without ritual suspicion, housing without preconditions, work without requiring perfect recovery, and diagnosis without identity foreclosure. None of this abolishes illness. It changes what illness is allowed to take.

The Romanian child who stopped crying teaches the final lesson. Human beings adapt to the world they are given. Adaptation can look like symptom, silence, defiance, withdrawal, vigilance, or dependence. The task

of science is to distinguish adaptation from disorder without romanticizing either. The task of institutions is to stop demanding that people become healthy inside conditions that repeatedly train them not to be.

Society is not the whole cause. It is part of the patient's environment, part of the treatment, and sometimes part of the disease. Once that is known, neutrality becomes a choice about which causal links we are willing to leave untouched.

Selected Sources and Further Reading

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